



Health Delivery System in Ethiopia

Agenda

- Introduction: Overview of the Ethiopian Health System
- Organization of Ministry of Health
- National Health Policy
- National Health Plan
- Specific Program Within the Health Service





Introduction

- ☐ What a Health System is
- ☐ WHO Health System Framework
- ☐ Ethiopia's Health System Framework
- ☐ Health Service Delivery
- ☐ Health sector envisioning and strategic planning
- ☐ Health Financing

Health System

- A health system **comprises all organizations, institutions and resources (elements) that are devoted to producing health actions.**
- The intrinsic goal of a health system is to protect and improve the health of the people, that is it is concerned with people's health.
- It is the sum total of all the organizations, institutions and resources whose primary purpose is to improve health.
- A health system needs staff, funds, information, supplies, transport, communications and overall guidance and direction.
- And it needs to provide services that are responsive and financially fair, while treating people decently.

Health System

- Overall, there is a growing recognition that to maintain and improve the health of the world's people governments must shape sound, efficient health systems that provide effective disease prevention and treatment to all women, men and children, no matter who they are or where they live.

Health System strengthening

- Strengthening health systems and making them more equitable are key strategies for fighting poverty and fostering development.
- Strategies, interventions and activities designed to sustainably improve health system performance

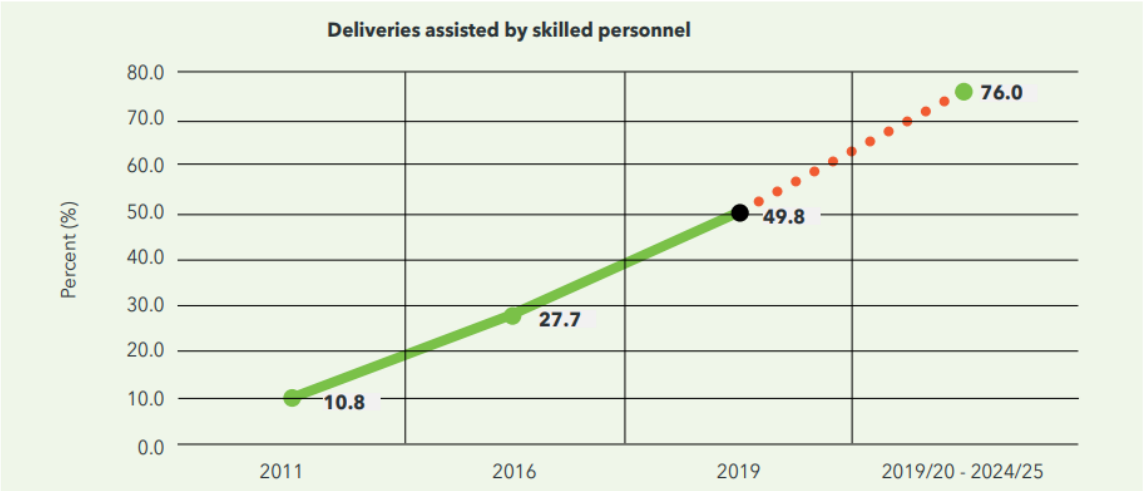
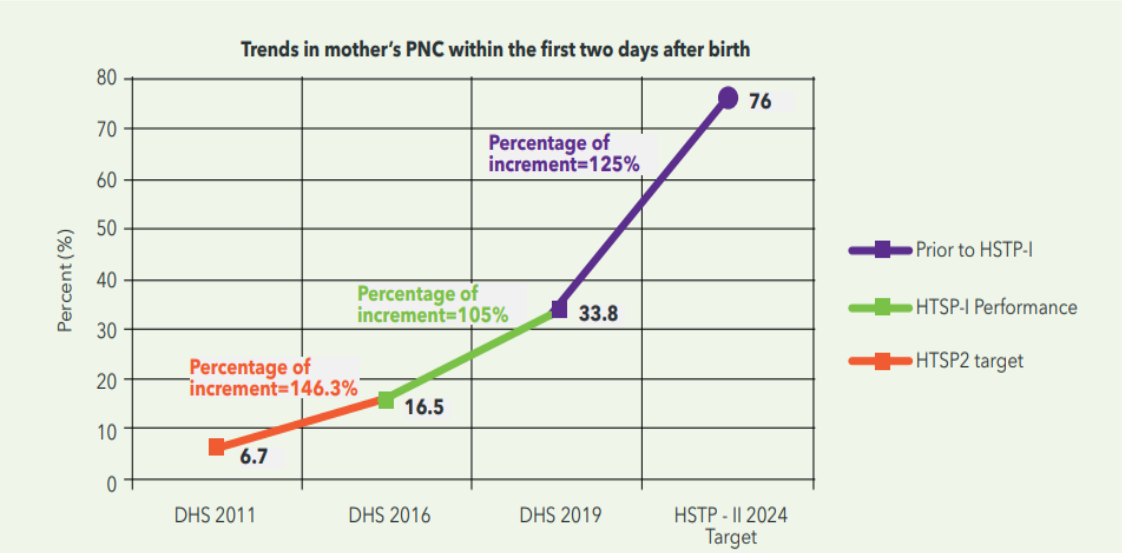
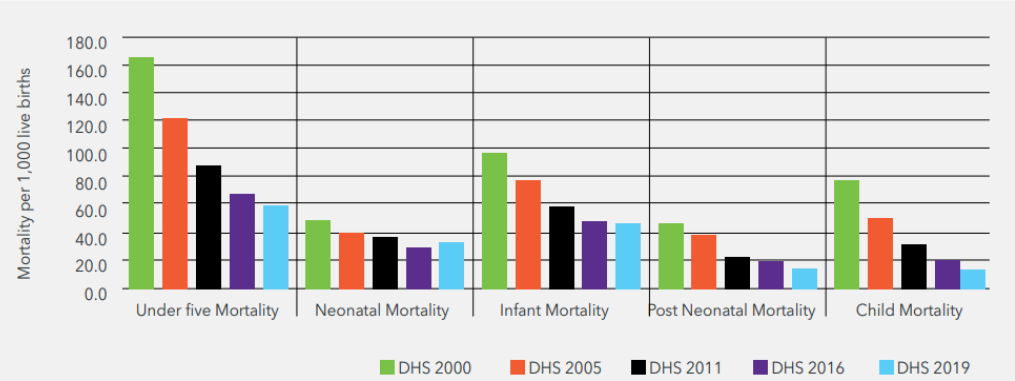
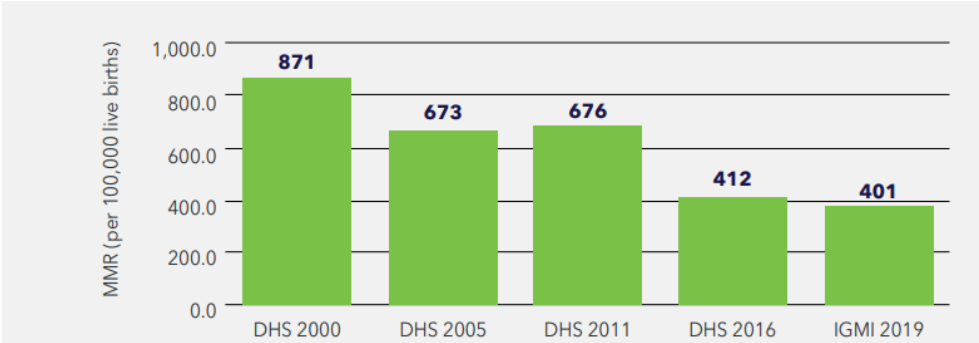


The Ethiopian Health Sector Development Programs and its achievements

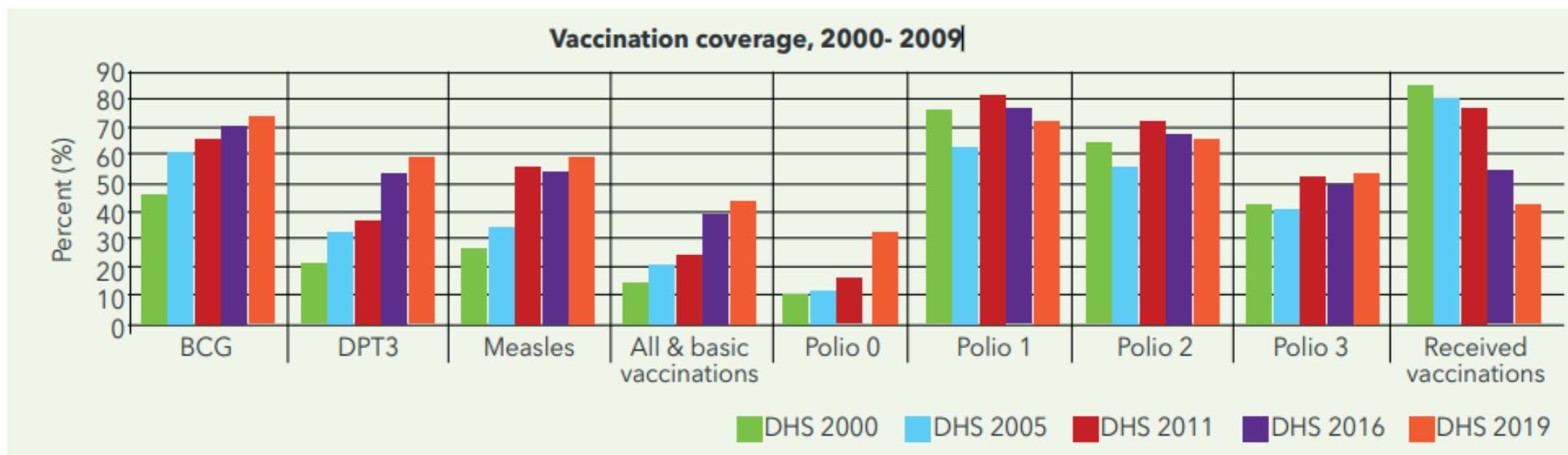
- ❑ The Ethiopian Health Sector Development Programs (HSDP) have been pivotal in shaping the country's healthcare landscape.
- ❑ HSDP I focused on establishing a solid foundation for health care services, emphasizing primary health care and disease prevention.
- ❑ HSDP II aimed to consolidate the gains of the first phase, with a strong emphasis on improving service delivery and quality of care.
- ❑ HSDP III continued this trajectory, further strengthening health systems and services, particularly in rural areas.
- ❑ Finally, HSDP IV sought to enhance the quality and equity of health services, aligning with the country's goal to become a middle-income nation by improving access to health services, community ownership, resource mobilization, and emergency preparedness. Each phase built upon the previous, with a vision of creating a comprehensive and resilient health system accessible to all Ethiopians.

Health outcomes	EDHS 2000	EDHS 2005	EDHS 2011	EDHS 2016
Key Health outcomes				
Maternal mortality ratio	871	673	541	412
Under five mortality rate	166	123	88	67
Infant mortality rate	97	77	59	48
Neonatal mortality rate	49	39	37	29
Stunting	58	51	44	38
Under-weight	41	33	29	24
Wasting	12	12	10	10
Total fertility rate	5.5	5.4	4.8	4.6
Essential interventions				
All basic vaccination (children 12-23 months)	14	20	24	39
Anemia (children 6-59 months)	56.9	54	44	57
Family planning (any vs modern method)	8,6	15,14	29,27	36,35
Skill birth attendance	6	6	10	28
ANC +4 coverage	10	12	19	32

Achievements



Achievements



Health System Shifts

Alma Ata 1978

PHC Renewal 2005

PHC Reform WHR 2008

Universal Health Coverage (UHC)

Our history: from PHC to Universal Health Coverage

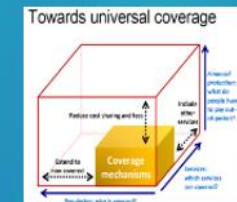
Alma Ata
1978



PHC Renewal
2005

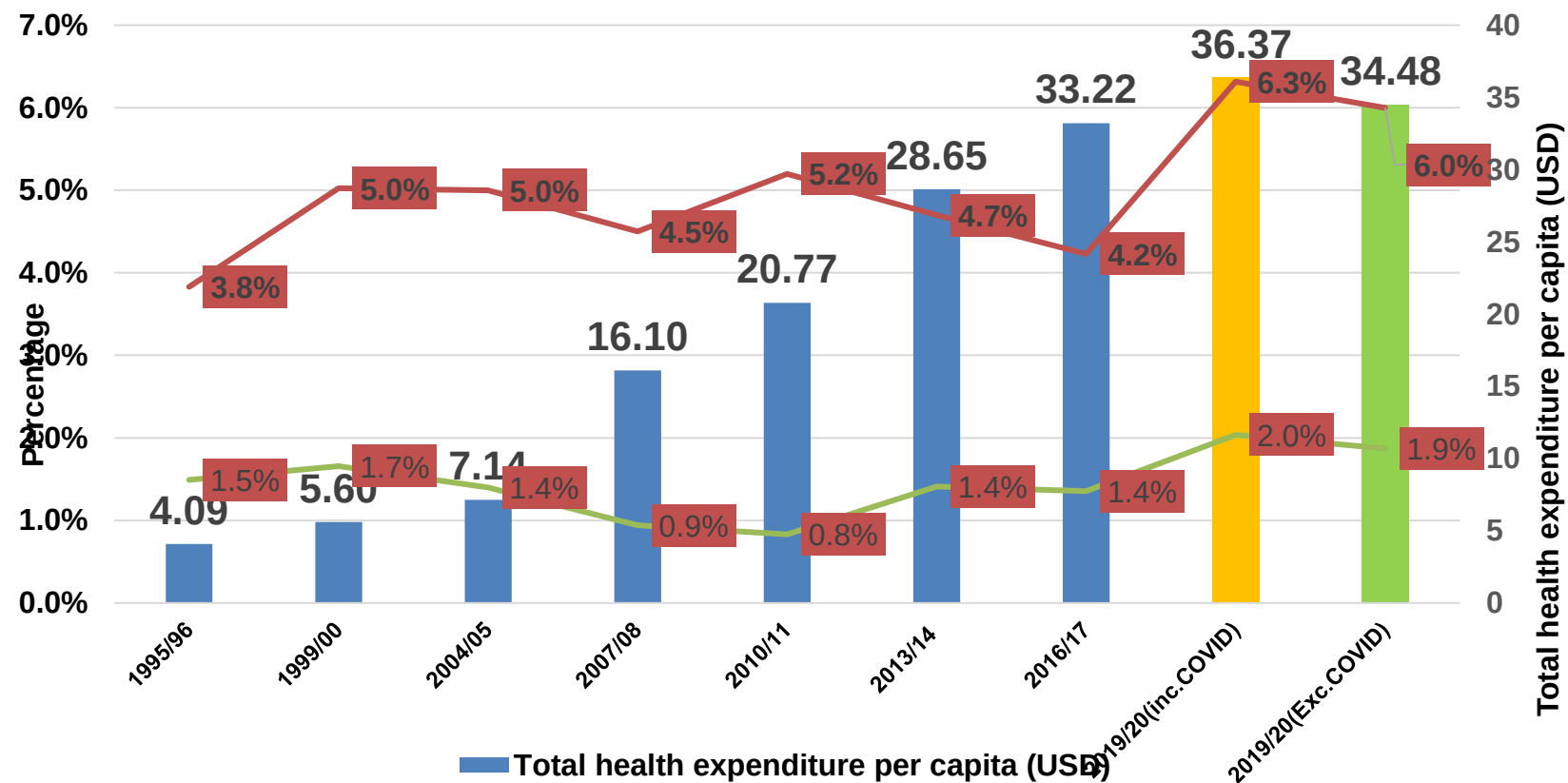


The PHC Reforms
WHR 2008



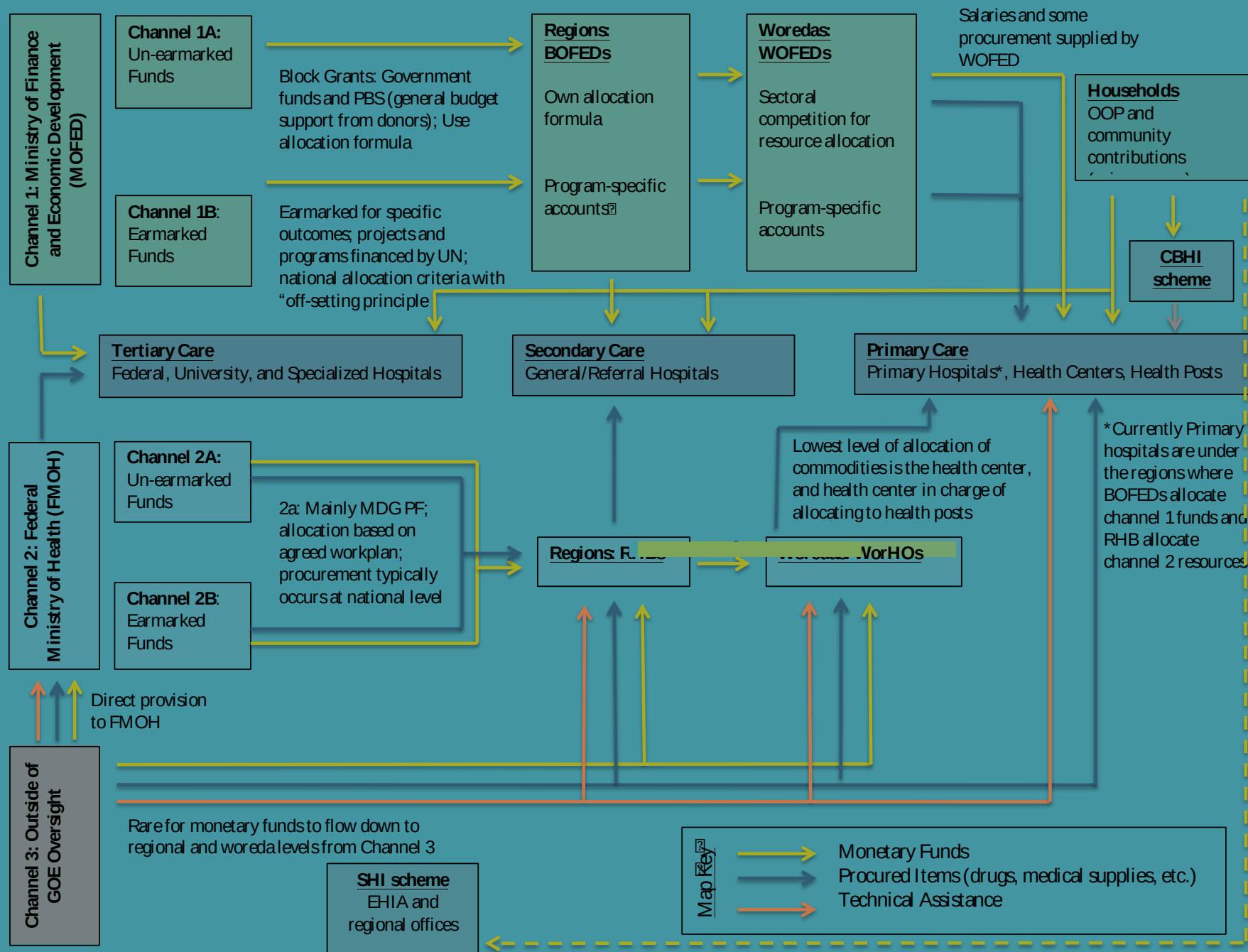
Per Capita Health expenditure): USD 36.30, of which USD 1.9 per capita spent on COVID-19

Per Capita Health expenditure (excluding Covid 19): USD 34.40



Total health expenditure per capita (USD),THE/GDP and THE/GGHE

Finance Sources



The HSTP aims to address the gaps in quality, equity, and sustainability of health services.

It focuses on transforming the health care delivery system, enhancing health service quality, and improving health outcomes through a more integrated and comprehensive approach to health system strengthening.

This strategic shift reflects Ethiopia's commitment to achieving universal health coverage and improving the health status of its population.

Health Sector Transformation I? why



Universal health Coverage (UHC)

Universal Health Coverage

Universal health coverage means that all people and communities have equitable access to the comprehensive, and guaranteed quality services that they need, throughout the life course, without financial hardship.

It addresses the social determinants of health emphasizes on groups in conditions of poverty and vulnerability.



Values

- ✓ Right to Health
- ✓ Equity
- ✓ Solidarity



Universal Health Coverage

- In January 2016, the UN adopted the pursuit of UHC as one of its 17 Sustainable Development Goals.
- The WHO defines UHC as “ensuring that all people can use the promotive, preventive, curative, rehabilitative and palliative health services they need, of sufficient quality to be effective, while also ensuring that the use of these services does not expose the user to financial hardship.”

The UHC “Cube”

Breadth of coverage

- Who is covered?

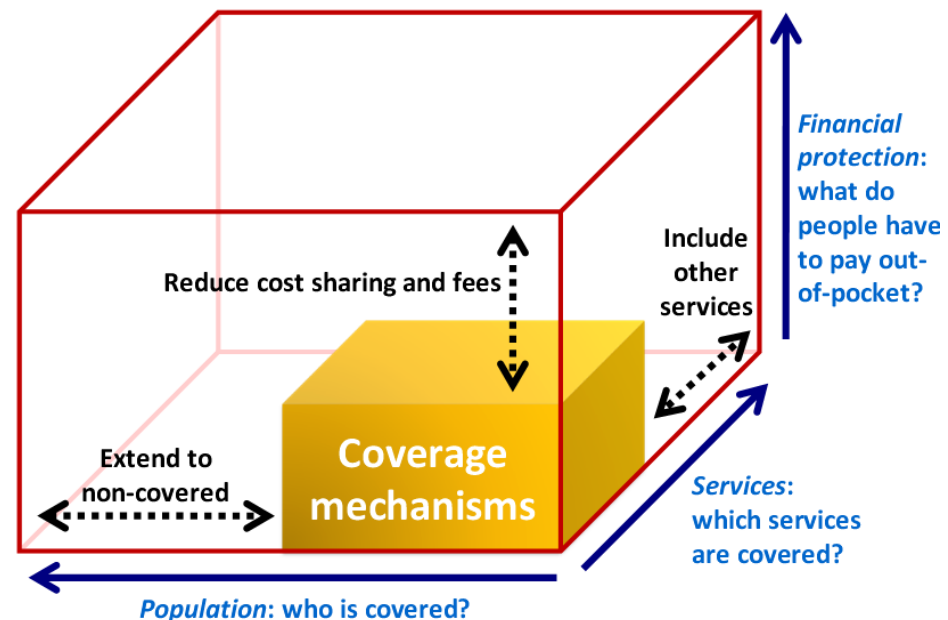
Scope of coverage

- What services are covered?

Depth of coverage

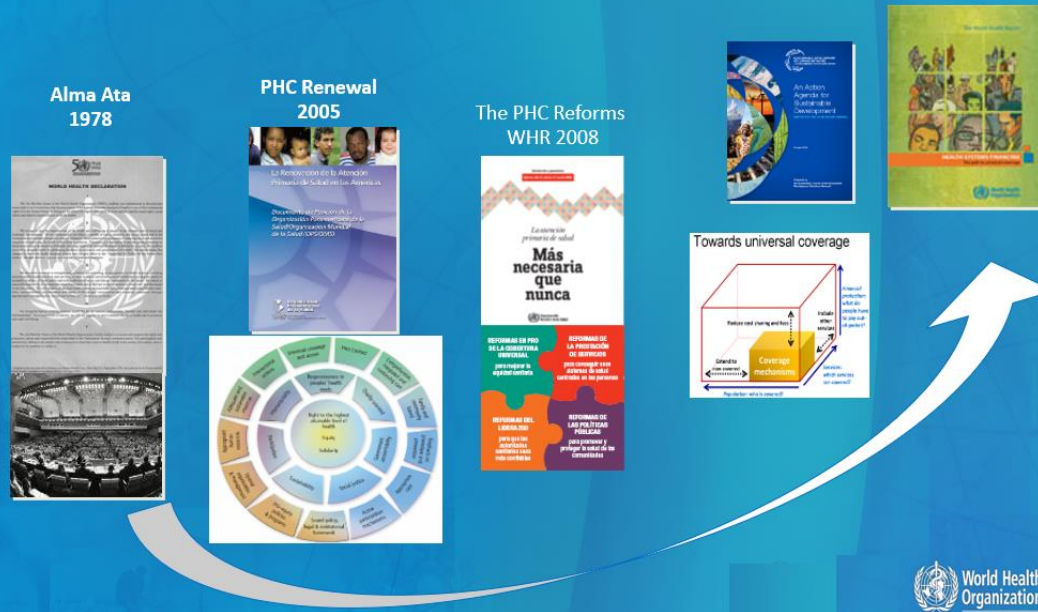
- To what extent are covered services funded?

Towards universal coverage

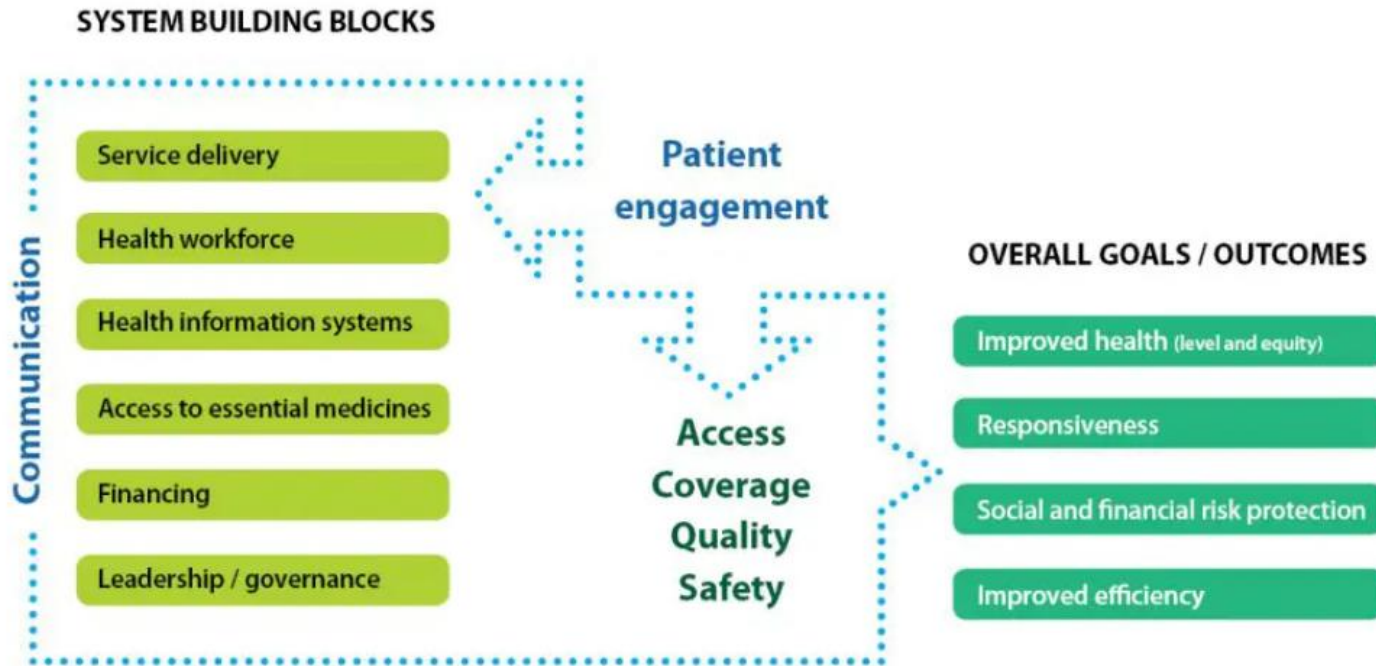


PHC and UHC

Our history: from PHC to Universal Health Coverage

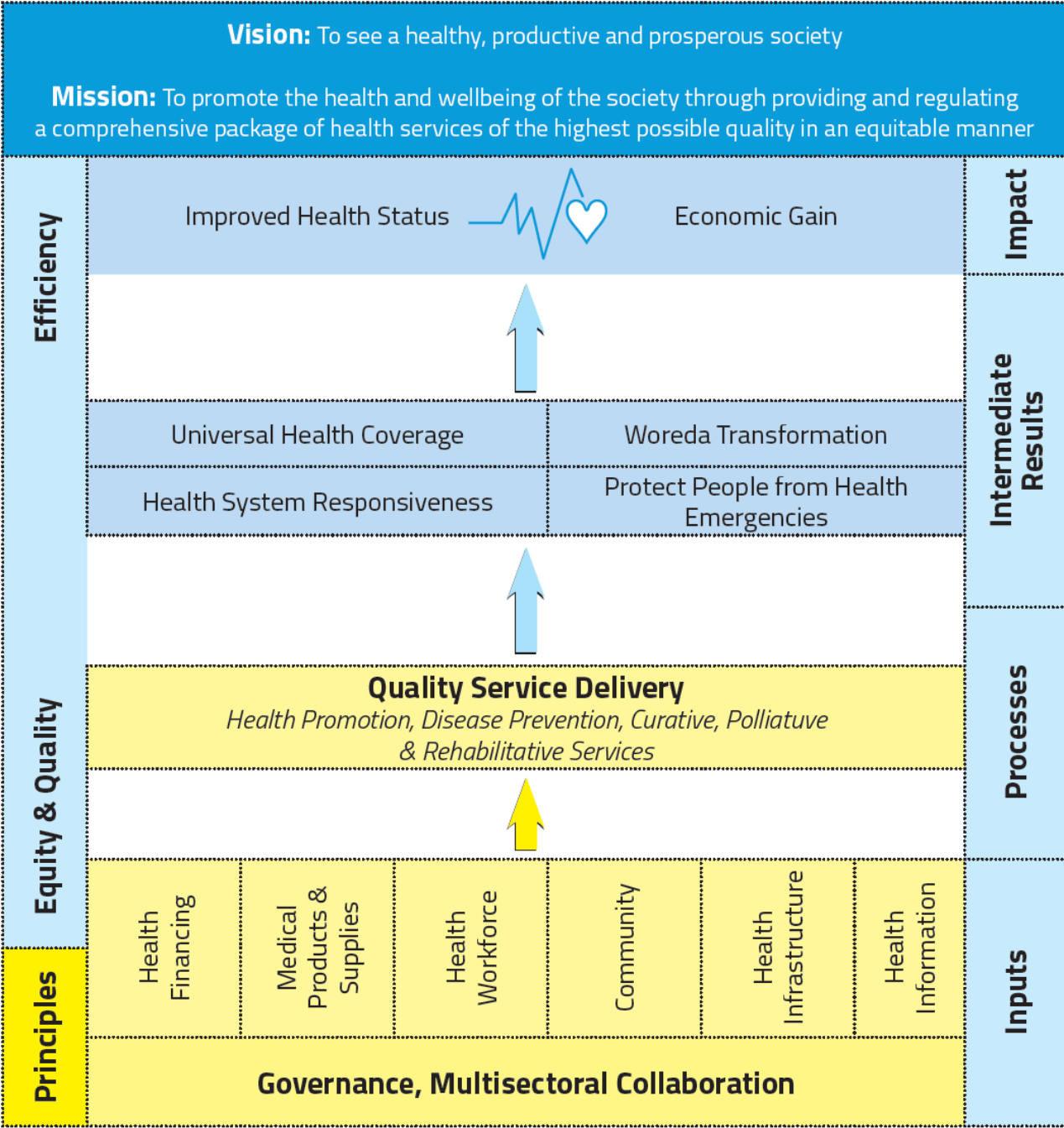


The WHO Health System Framework

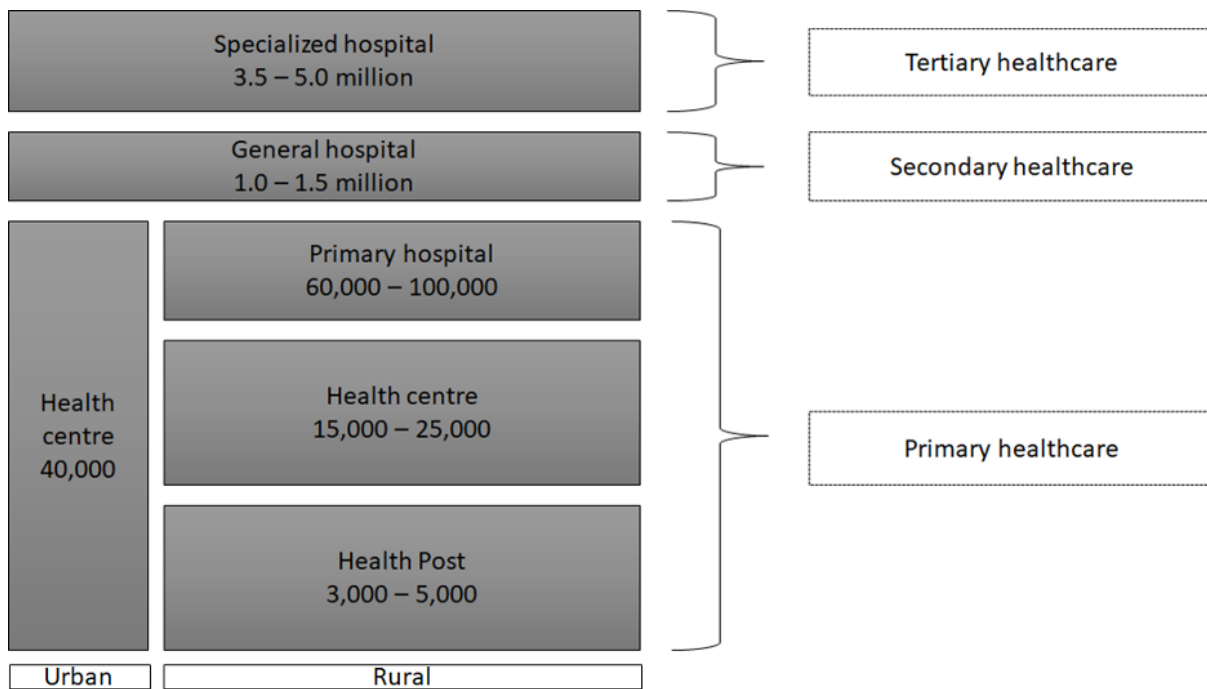


Ethiopia's Health System Framework

Ethiopia's



Health Service Delivery



- Three tier system

Visioning and Strategic Planning

- The main goal of the health system is ensuring that everyone who needs health services (promotion, prevention, curative, rehabilitative and palliative services) is able to get them without undue hardship.
- Hence, UHC needs to be a goal for Ethiopia's health sector in the coming years
- UHC has been defined as guaranteeing access to all necessary services for everyone while providing protection against financial risk
- As Ethiopia advances to middle income country status, its goal is to progress towards UHC and ultimately, achieve UHC for all Ethiopians.
- Strong investment in PHC are anticipated to result in continued improvements in health outcomes, building on gains seen since the launch of the health extension program.

Visioning and Strategic Planning

- Over the last decades, Ethiopia has made great improvements in many health indicators, due in large part to a well-coordinated, extensive effort and intensive investment of the government, partners and the community at large in primary care through the health extension program and expansion of PHC units.
- It is a priority of the ministry of health to expand and sustain this progress which will require careful visioning and strategic planning for the future health care system
- Over the last couple of years the ministry of health has engaged in a vision exercise to think broadly and strategically about the long term development of the Ethiopian primary health care system.

Visioning and Strategic Planning



- The HSTP 1 is the first phase of a 20 year plan titled “Envisioning Ethiopia’s path to Universal Health Care through Strengthening of Primary Health Care”
- The purpose of the visioning exercise was to develop a system that will ensure quality health services and be equitable, sustainable, adaptive and efficient to meet the health needs of a changing population between now and 2035
- It is anticipated that, Ethiopia will continue the past pace of development and will transition into lower-middle income country by 2025 and middle-middle income country by 2035

HSTP - 1

The strategy focused on how the health sector intends to achieve its mission and vision for its communities and stakeholders.

E.H.S. Strategic Themes

1. Excellence in health service delivery
2. Excellence in Quality Improvement and Assurance
3. Excellence in leadership and good governance
4. Excellence in health system capacity

HSTP - 1

CORE VALUES

1. Community first
2. Integrity, loyalty, honesty
3. Transparency, accountability, confidentiality
4. Impartiality
5. Respecting the law
6. Be a role model
7. Collaboration
8. Professionalism
9. Change/Innovation
10. Compassion

GUIDING PRINCIPLES

1. Self-reliance
2. Community ownership
3. Universal health coverage
4. Focus on Primary Health Care
5. Patient/client centered quality health service
6. Equity, pro-poor and affordability
7. Good governance
8. Gender equality, youth focused
9. Participatory partnership
10. Learning institution/system
11. Professional ethics
12. Continuous professional development

Transformation Agenda

1. Transformation in equity and quality of health care
2. Information revolution
3. Woreda Transformation
4. The Caring, Respectful and Compassionate health workforce

HSTP - 1 Performance



Health status	Under five mortality (per 1,000 live births)	64	30	59
	Infant mortality (per 1,000 live births)	44	20	47
	Neonatal mortality (per 1,000 live births)	28	10	33
	Stunting (under five years old)	40	26	37
	Wasting (under five years old)	9	4.9	7
	HIV incidence rate (per 1,000)	0.03	0.25	0.16
	TB incidence rate (per 100,000)	224	156	151
	TB mortality rate (per 100,000)	32	17	22
	Malaria death rate (per 100,000)		0.6	0.32
	Contraceptive prevalence rate	42	55%	41%
Status of Priority Health Programs	Unmet need for FP	24	10%	22%
	ANC (at least four visits)		95%	43%
	Skilled birth attendance		90%	50%
	Postnatal care coverage		95%	34%
	Still birth rate (per 1000 live births)	18	10	15
	CS rate as proportion of births	2.2	8	4
	ART to prevent MTCT of HIV	59%	95%	91%
	Three doses of pentavalent immunization		98%	61%
	Exclusive breast feeding at six months		72%	59%
	ART coverage – Adult >= 15 years	82%	90%	75.8%
	TB case detection rate	61%	87%	71%
	TB treatment success rate	92%	95%	95%
	Proportion of women screened for cervical Ca	0.6%	20%	5%
	% of HHs with safe water source		35%	28%
	OPD attendance per capita	0.48	2	0.9
Health Security	Average length of stay	4.3	5 days	4.5 days
	Health emergency risk management			
	Proportion of emergency affected people provided rehabilitation	36%	95%	84%
Health System	Proportion of epidemics controlled	NA	85%	80%
	Health workers per 1,000 population	0.84	1.6	1.74
	Proportion of Woredas with CBHI	15%	80%	70%
	OOP as a proportion of THE	33.7%	15%	31%
	% of HHs with REmQMS	56%	100%	71%

HSTP II with vision, goals, objectives and strategic direction

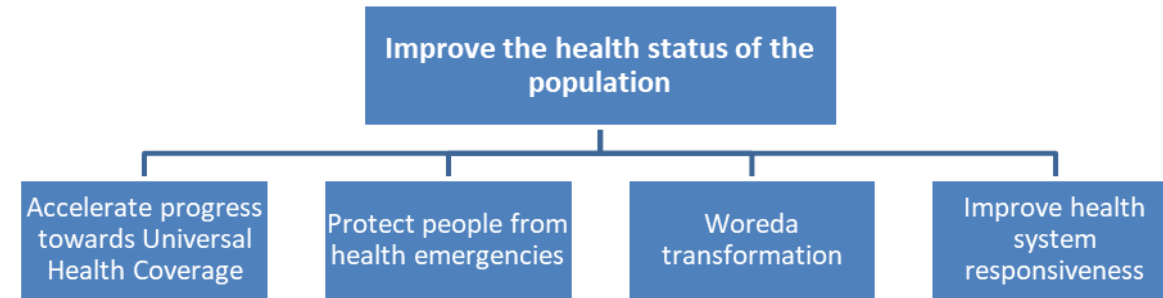
Vision To see healthy, productive and prosperous society

Objectives

- Five objectives
- Has 14 strategic directions
- Each strategic direction has specific interventions
- Has 76 indicators of which 6 of them are health care financing indicators
- Has five priorities/transformation agendas
- Organized in seven chapters

General indicators

1. Increase life expectancy at birth from 65.5 to 68
2. Increase UHC index from 0.43 to 0.58
3. Increase proportion of clients satisfied during their last healthcare visit (Client satisfaction rate) from 46% to 75%



Strategic directions

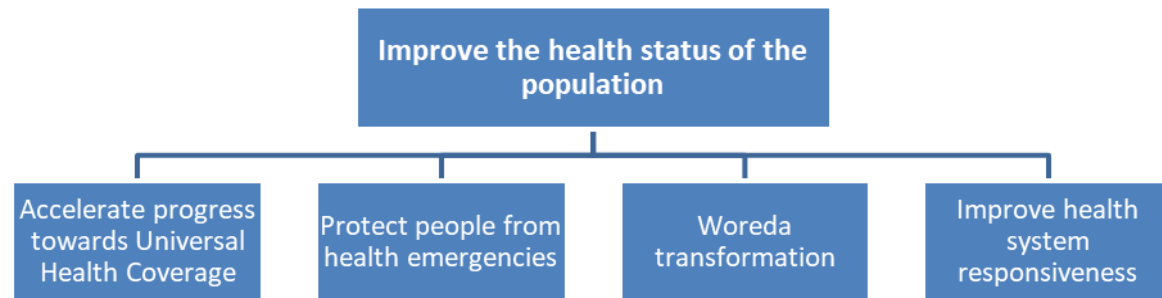
1. Enhance provision of equitable and quality comprehensive health services
2. Improve health emergency and disaster risk management
3. Ensure community engagement and ownership
4. Improve access to pharmaceuticals and medical devices and their rational and proper use
5. Improve regulatory systems
6. Improve human resource development and management
7. Enhance informed decision making and innovations
8. Improve health financing
9. Strengthen governance and leadership
10. Improve health infrastructure
11. Enhance digital health technology
12. Improve traditional medicine
13. Ensure integration of health in all policies and strategies
14. Enhance private engagement in the health sector

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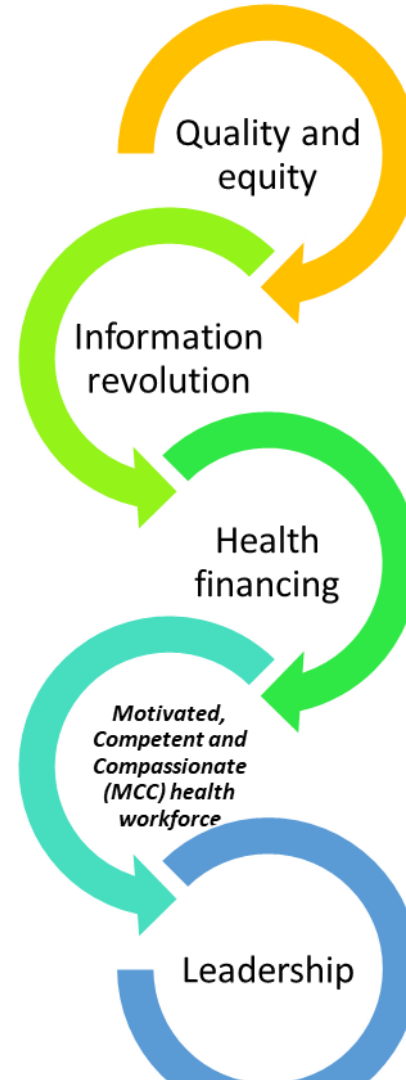
HSTP II

Priorities /Transformation Agendas of HSTP-II

Quality and equity: ensuring delivery of quality health care (reliable, patient-centered and efficient) to all in need in an equitable and timely manner. Regarding equity, regular monitoring of state of equity at all levels of the health sector will be conducted and tailored interventions, such as redesigning mode of service delivery, incentive package and others, will be designed and implemented.

Information Revolution: Improving the capability of the health system to generate and use high quality data for evidence-based decision-making. Transforming culture of high quality data use, digitization of health information system (HIS) and improving HIS governance.

Health financing: proactive mobilization of adequate resources from domestic and international sources, reforming resource allocation & prioritization, optimization of the health insurance system, public-private partnership, reforming the cost recovery mechanisms, implementing performance based financing and design & implement strategies for efficient use of resources and capacities



MCC:

This priority area/transformation agenda refers to ensuring the availability of adequate number and mix of quality health workforce that are Motivated, Competent and Compassionate (MCC) to provide quality health service.

Leadership:

redesigning & restructuring the health system, institutionalize accountability mechanisms, strengthen clinical governance, ensuring regulatory system autonomy, and strengthen stakeholder engagement and partnership, building leadership capacity at all levels and ensuring health all policies & strategies.

How do we understand the health systems? Understanding the health sector: Methods

- ❑ First approach is the WHO building blocks (WHO, 2000);
- ❑ The control knobs: WB approach: Getting Health Reform Right: A Guide to improving Performance and Equity (Roberts et al, 2008
- ❑ Second approach is that adopted sociologist (Roemer, 1993; Raffel, 1984):
- ❑ A third approach has focused on resource flows (Hurst 1992);
- ❑ A fourth approach is by economist (Jack 1999; Feldstein, 1993; Feldstein, 1994; Yett et al, 1973)

Let us see the two cases and focus on the control knobs: WB approach.

WHO “Building Blocks” for achieving the UHC

THE WHO HEALTH SYSTEM FRAMEWORK

SYSTEM BUILDING BLOCKS

SERVICE DELIVERY

HEALTH WORKFORCE

INFORMATION

MEDICAL PRODUCTS, VACCINES & TECHNOLOGIES

FINANCING

LEADERSHIP / GOVERNANCE

ACCESS
COVERAGE



QUALITY
SAFETY

OVERALL GOALS / OUTCOMES

IMPROVED HEALTH (LEVEL AND EQUITY)

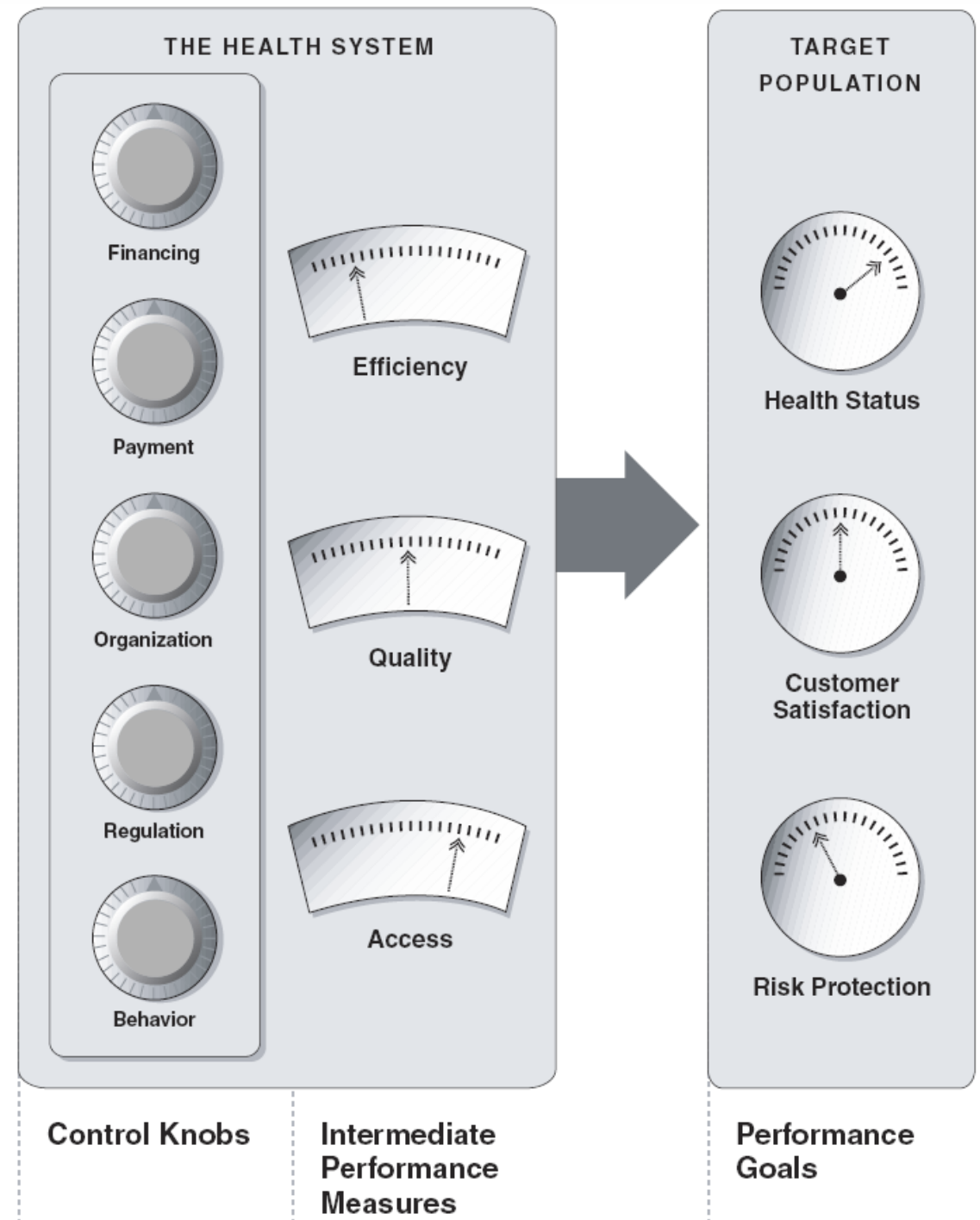
RESPONSIVENESS

SOCIAL AND FINANCIAL RISK PROTECTION

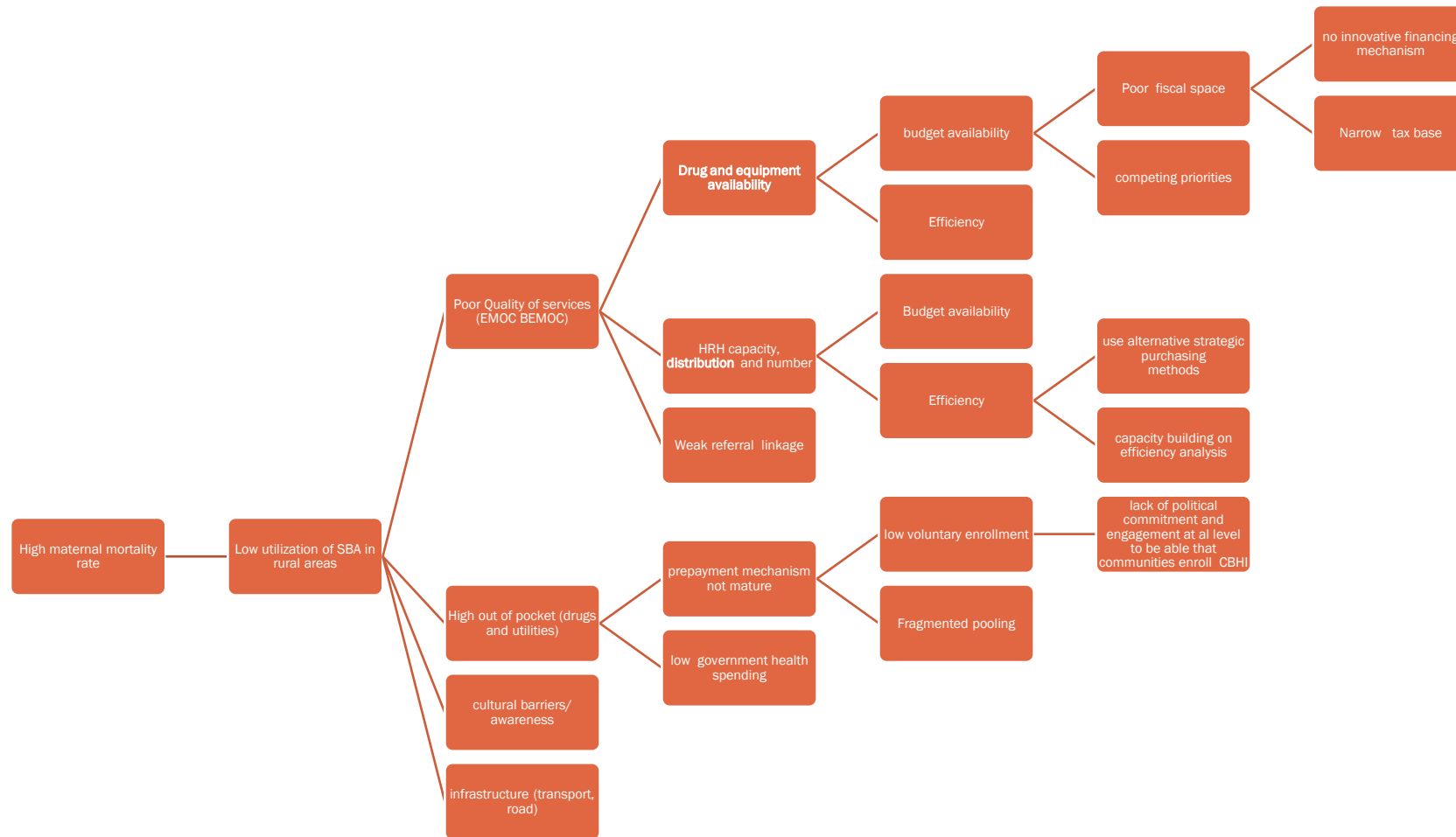
IMPROVED EFFICIENCY

World Bank Control knob Approach for achieving the UHC

- Ethics and Politics (through out implementation cycle)
- ✓ Problem definition
- ✓ Diagnosis
- ✓ Policy development
- ✓ Political Decision
- ✓ Implementation
- ✓ Evaluation



Identification of Issues: Implementation Cycle



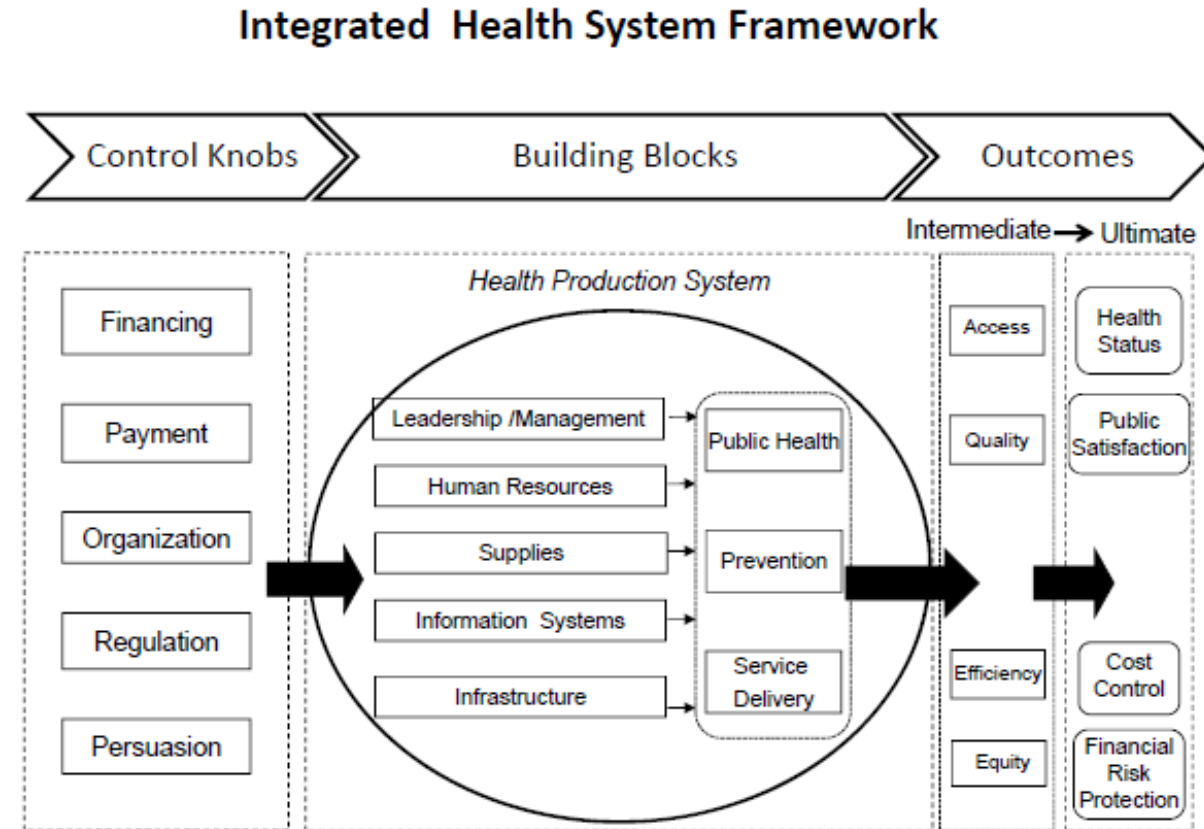
The Health System Actionable Areas “Control Knobs”

- Financing—How funds are collected and organized
- Payment— How those funds are transferred to doctors, hospitals and other providers
- Organization—Both the macro aspects of “who does what” and the micro aspects of internal managerial structures
- Regulation— Requirements on behavior imposed by the state
- Persuasion—Efforts to influence the behavior of both providers

Integrated Health System Framework

➤ WHO Building Blocks is an alternative framework which has some similarities but is more static and less oriented to change.

➤ Both approaches can be complementary

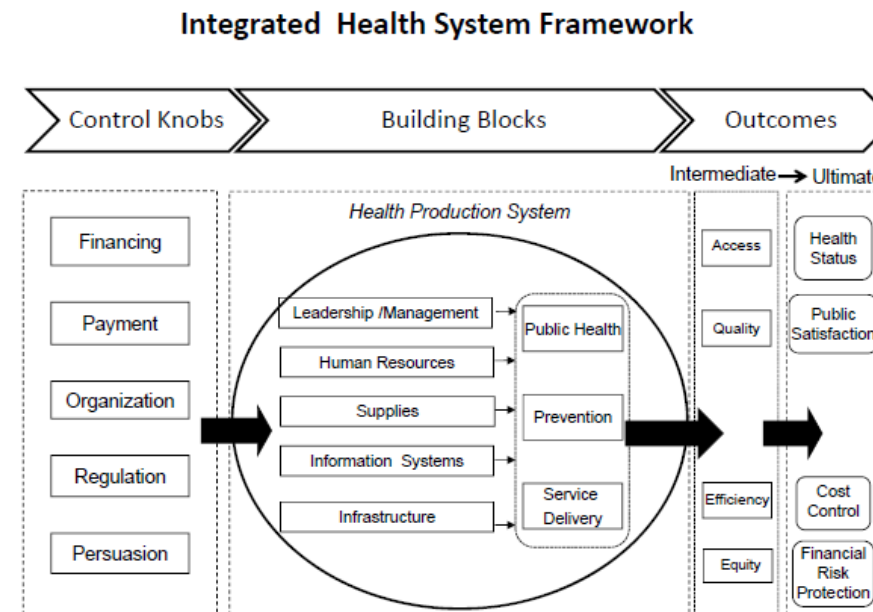


What are Key Performance Problems?

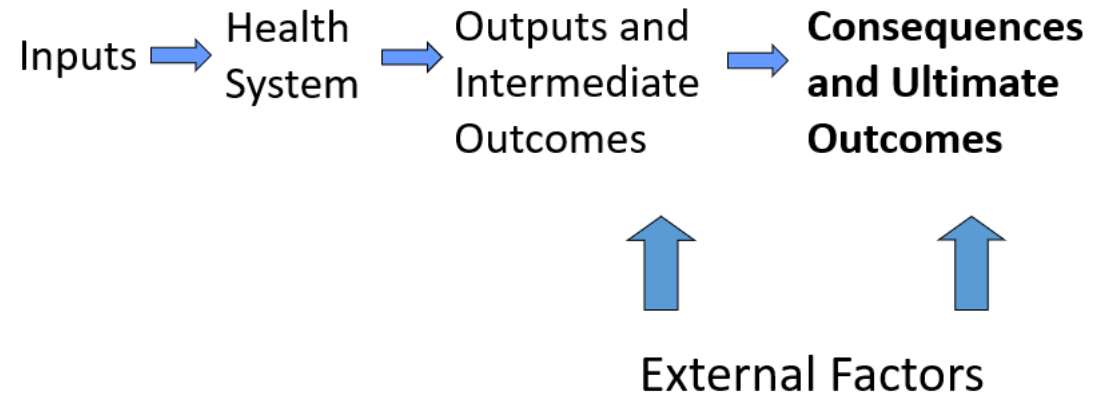
Three Ultimate Objectives (performance outcomes)

- **HEALTH STATUS IMPROVEMENT** – reduce causes of death and morbidity
- **CITIZEN SATISFACTION** – improve patient and citizen satisfaction with the health system
- **FINANCIAL RISK PROTECTION** – reduce the financial cost to individuals of getting sick and using the health system

Need to have **objective measures** (metrics) for each



Basic Flow Diagram for the Health System





Thank you very much

Any questions, suggestions and comment
sare welcome



Methods of Economic Evaluation of health care programs

Meseret Molla Kassahun, PhD

Methods of Economic Evaluation

- ❑ Economic evaluation of healthcare programs is a critical tool for decision-makers, involving the comparison of the relative costs and outcomes of different courses of action.
- ❑ Key methods include cost-effectiveness analysis, which compares the cost per unit of health outcome; cost-utility analysis, which takes into account the quality and quantity of life gained; and cost-benefit analysis, where benefits and costs are expressed in monetary terms.
- ❑ These methods help in understanding the value of healthcare interventions and in making informed choices about resource allocation in healthcare systems.
- ❑ For a comprehensive understanding, texts such as "Methods for the Economic Evaluation of Health Care Programmes" provide detailed insights into these methodologies.



Economic Evaluation

Meseret Molla Kassahun, PhD



Economic evaluation

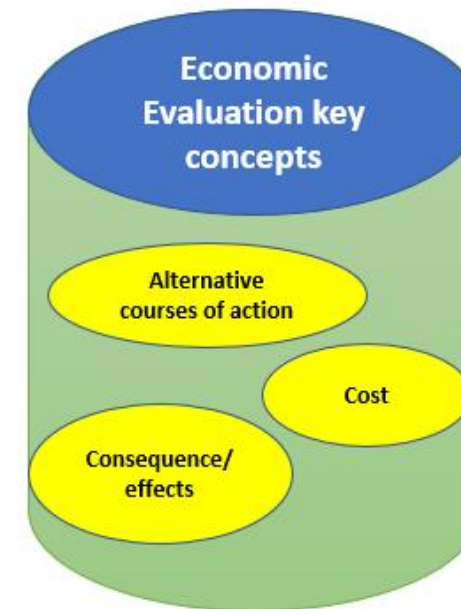


- Economic evaluation explicitly incorporates concept of **opportunity cost** i.e. the value of what has to be sacrificed in order to obtain the next best alternative option.
- Economic Evaluation is a comparative analysis of **alternative courses of action** in terms of both **costs** and **consequences** (Drummond et al, 2005)

Economic Evaluation

Economic evaluation

- Aims to identify best use of available resources to maximise health benefits
- EE is now increasingly been recognised as crucial to realising the benefits of **UHC** by enabling efficient and equitable allocation of health care resources.



Cost

Estimates the total cost of a program

- Labor: wages, training
- Materials
- Overhead (administration costs)

Typically reported as

- Cost per patient
- Cost per service provided

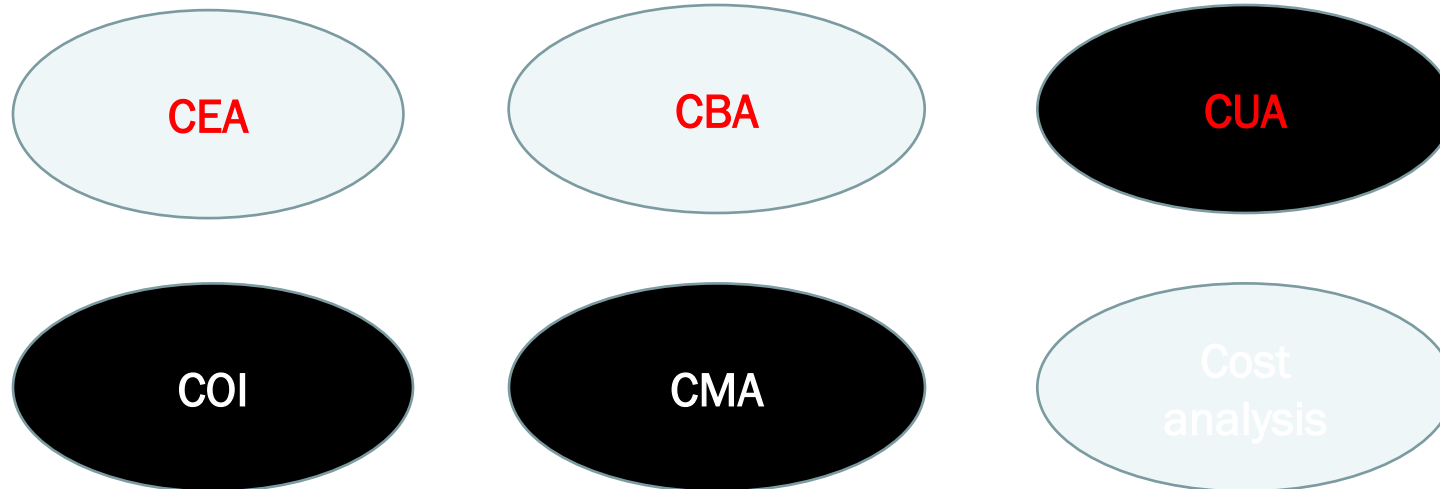
Primary uses

- Budgeting
- Accountability
 - Estimates the costs of a program
 - Assesses the efficiency of a program
- Basis for a full economic evaluations

ECONOMIC EVALUATION

To analyze the economic contribution, it is critical to apply economic evaluation

- Economic evaluation is defined as *the comparative analysis of alternative courses of actions both in terms of cost and consequences* (Drummond et al., 2005).



Cost-benefit Analysis



- ❑ CBA is the most comprehensive and arguably the most theoretically sound form of economic evaluation. Unlike the other approaches we shall consider, CBA places monetary values on both the inputs (costs) and the outcomes (benefits) of any project to which it is applied.
- ❑ CBA is used both to determine allocative efficiency and to answer cost-efficiency questions.
- ❑ Allocative efficiency assesses competing programmes and judges the extent to which they meet objectives.
- ❑ Cost-efficiency assesses the least costly way of achieving a given objective.

Cost-benefit Analysis



- ☐ CBA can help to minimize the uncertainty when deciding how and where to allocate scarce resources. Since it is a quantitative analytical tool, it will aid decision makers in the efficient allocation of resources.
- ☐ The aim of CBA is to determine whether a project is desirable from the point of view of social welfare, by means of the algebraic sum of the time-discounted economic costs and benefits of the project (Commonwealth of Australia, 2006).
- ☐ Discounting is also an issue here

Cost-benefit Analysis

- ❑ The World Health Organization (2006) identifies the following steps as required for a CBA in the field of health:
 - define the objective of the intervention
 - define the baseline
 - define the alternative options to achieve the objective
 - quantify the investment costs of each option compared to the baseline 5. identify the positive and negative welfare effects of each alternative option compared to the baseline
 - value the welfare effects in monetary terms, using market prices and economic valuation methods
 - calculate the present value of costs and benefits occurring at different points in time using an appropriate discount rate 8. calculate the net present value (NPV) or benefit/cost (B/C) ratio of each alternative option
 - perform a sensitivity analysis
 - select the most efficient intervention option

Cost-effectiveness analysis

- ❑ CEA compares the costs of achieving an agreed objective through alternative interventions or the alternative potential benefits that can be achieved with a given level of resources.
- ❑ Inputs are, as in CBA, measured (expressed) in monetary terms, but benefits, or outcomes, are not monetised.
- ❑ As we have seen, some benefits of a project or policy are very difficult to measure in monetary terms, so we may simply take the physical quantities, measured in natural units, such as health improvement, lives saved, life years gained, number of people treated or reduction of the incidence of a disease as outcomes in terms of effectiveness.
- ❑ On the basis of a CEA we can select the possible maximum benefit obtainable with a given level of resources or the minimum cost route to achieving given benefits. Either way, we are concentrating on technical efficiency

Cost-effectiveness analysis

- ❑ CEA is particularly useful in evaluating different methods of addressing a single disease problem, but care should be taken to ensure that lower costs do not mean loss of quality.
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- ❑ The natural units in which benefits are measured will depend on the type of benefits under consideration. The scale may be either a clinical parameter, such as the lowering of blood pressure by a certain amount, or a simple numerical one, such as the number of additional years of life or the decrease in the number of deaths attributed to the intervention.
- ❑ CEA relates costs to a single clinical or natural measure of effectiveness; i.e. a one-dimensional outcome, such as pain reduction or life years gained.
- ❑ It does not show the multiple effects or consequences of a particular intervention.

Cost-effectiveness analysis

- ❑ Comparisons can be made between different health programmes with common effect in terms of their cost-effectiveness ratios: that is, their cost per unit of effect.
- ❑ The average cost effectiveness ratio (ACER) is an index used to make comparison of the costs and consequences or benefits of independent interventions, i.e. interventions the costs and benefits of which are not affected by other interventions. For example, if effectiveness is measured by length of life, ACER it is defined as:
$$\checkmark \text{ACER} = \frac{\text{costs in units of money}}{\text{benefit in life years gained}}$$

Cost-effectiveness analysis

Suppose that we have a 1 million birr budget to be used for a healthcare project. The alternative outcomes from different uses of this budget are:

1. used for control of Malaria, it can save 10,000 lives per year
2. used for HIV/AIDS programme, it will reduce the stigma against HIV virus by 60%
3. used for vaccination, it will improve life expectancy of a given nation by 10%
4. used to treat heart attacks, it can save 100 lives per year
5. used to improve child nutrition, it can reduce child mortality by 40%.

Which of these projects can be compared using CEA?

Comment

Only projects 1 and 4 are comparable by CEA, because in both the objective is the number of life saved per year. All the others have different objectives. Clearly, project 4 is preferred to project 1 as the cost per life saved is 100 birr for 1 whereas it is 10,000 birr for 4.

Suppose we have a health problem affecting 100 people that can be treated by one of three healthcare interventions A, B and C. The costs and benefits obtainable from each are shown in Table 1.

Compare the ACER values of these interventions and complete the final column. Which intervention would be preferred according to CEA?

Intervention	Cost	Gain in life years	ACER – cost per life year gained
A	30,000	30	
B	50,000	40	
C	60,000	50	

Table 1 Costs and benefits of alternative interventions

Comment

Intervention	Cost	Gain in life years	ACER – cost per life year gained
A	30,000	30	10,000
B	50,000	40	12,500
C	60,000	50	12,000

Table 2 Cost per life year gained of alternative interventions

Intervention A will be preferred as it has the lowest ACER – that is, the lowest cost per life year saved.

Cost–utility analysis

- ❑ CUA can be seen as a variant of CEA.
- ❑ It is a form of economic study design in which interventions that produce different health related consequences, in terms of both 'quantity' and 'quality' of life, are expressed as 'utilities'.
- ❑ These are measures that comprise both 'quantity' (life years) and 'quality' of life gained from healthcare interventions.
- ❑ It is a more sophisticated and multidimensional method of evaluation.
- ❑ The best-known utility index used in CUA is the 'quality adjusted life year' or QALY.
- ❑ It is used to reduce or summarise all health related consequences/outcomes of healthcare intervention into a single index.
- ❑ QALY captures all aspects of health-related consequences of healthcare intervention by aggregation of the weighted utilities or consequences.
- ❑ Some of these consequences use contingent valuation techniques and methods used in CBA.

Cost–utility analysis

- ❑ To derive QALYs, all conceivable health states are evaluated and weighted on a scale running from 0 to 1.
- ❑ The worst health state (could be death) is assigned the value 0 and the perfect health state is assigned the value 1.
- ❑ The intermediate states are scaled in the range between the two extremes.
- ❑ Each weighted health state is multiplied by the expected duration – the number of years a particular health state will last.
- ❑ In this way, all health effects of an intervention are measured and weighted.
- ❑ They are then aggregated into a single number to obtain ‘gain in QALYs’.
- ❑ The concept of QALYs allows us to compare changes in the quality of life and in the length of life.

Cost-utility analysis

- ❑ The QALY score of an intervention provided for a given individual will of course depend on the age and life expectancy of the individual receiving the intervention.
- ❑ An advocate of the QALY approach would favour providing a given intervention with known health benefits in terms of quality to a younger rather than an older person as the younger person is likely to enjoy the benefits for a greater number of years.
- ❑ This preference for treating, say, a 25-year-old rather than, say, a 70-year-old person when the two have otherwise similar claims is sometimes referred to as the 'fair innings' argument.
- ❑ The older person is deemed to have had a 'fair innings' whereas the younger one has potentially many years of life to look forward to.

Cost–utility analysis

- ❑ QALY is a single index that captures all aspects of health related consequences of healthcare intervention.
- ❑ Competing interventions and competing candidates for treatment can be compared or ranked in terms of cost per utility index (cost per QALY).
- ❑ As a result, ‘cost per QALY gained’ calculations are often produced to compare the relative efficiency with which different interventions can turn resources invested into QALYs gained.
- ❑ The ratio used is known as the average cost utility ratio (ACUR) and is given by:
 - ✓ $ACUR = \text{cost of intervention} / \text{benefit in QALYs}$

Cost–utility analysis

- ❑ It is possible to compare surgical, medical, pharmaceutical and health promotion interventions with each other.
- ❑ Comparability then is the key advantage of this type (CUA) of economic evaluation over that of CEA.
- ❑ For a decision maker faced with allocating scarce resources between competing claims, CUA is potentially informative.

Cost–utility analysis



- ❑ CUA is concerned with technical efficiency and allocative efficiency within the healthcare sector.
- ❑ It can be thought of as a sophisticated form of CEA, since it also makes comparisons between different health programmes in terms of cost-effect ratios.
- ❑ Alternatively, it can be thought of as an alternative to CBA which instead provides a nonmoney-based common metric to compare projects.
- ❑ However, CUA differs from CEA in that it considers multiple effects. CUA is a multidimensional form of analysis whereas CEA is unidimensional.

Cost-utility analysis



- ☐ Note also that although both CEA and CUA provide a rank order of interventions they do not specify a cost-utility ratio which should be seen as the maximum acceptable for an intervention to be carried out. Such a question can be answered only in the presence of a fixed budget or when the healthcare budget is already known.
- ☐ CUA is mostly used when quality of life is an important factor involved in the health programmes being evaluated.
- ☐ This is because CUA combines life years (quantity of life) gained as a result of a health programme with some judgment on the quality of those life years. It is this judgment element that is labelled utility.
- ☐ CUA is a useful technique when the intervention under consideration has multiple effects and we are interested in capturing all health-related outcomes.



Thank you

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Health Care Financing in Ethiopia: Strategy, Key Achievements and Challenges

- By Meseret Molla Kassahun, PhD

Outline of the presentation

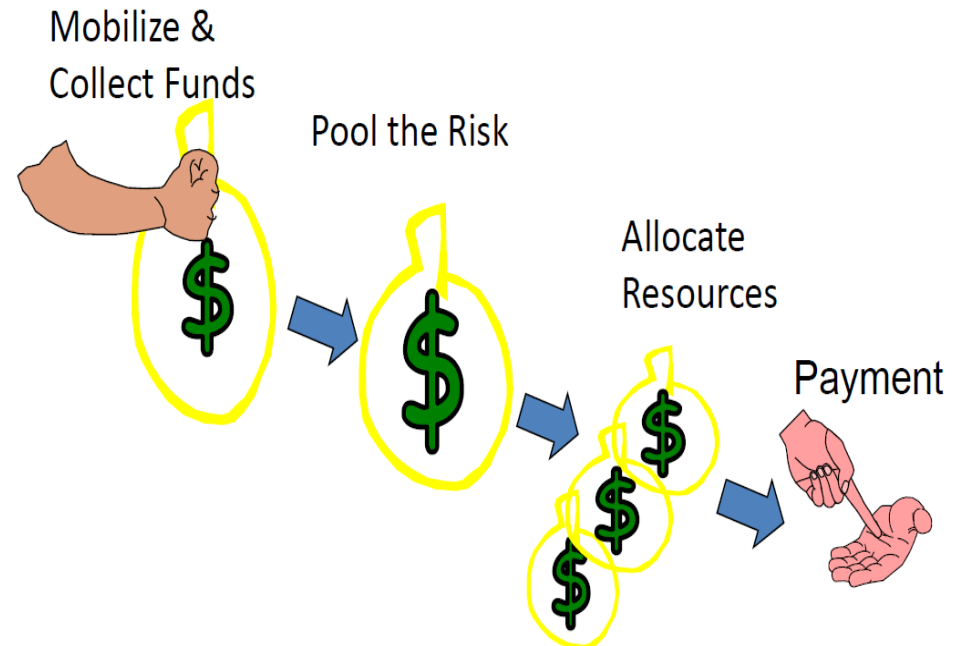
- ☐ Introduction;
- ☐ Evolution of HCF in Ethiopia;
- ☐ Overview of the 1998 Healthcare and Financing Strategy;
- ✓ Goals
- ✓ Strategies
- ✓ Principles
- ✓ Reforms
- ☐ Key Achievements and Challenges;
- ☐ Lesson Learned;
- ☐ Recommendation;
- ☐ Rational for Revising the Health care Financing Strategy.

Introduction: What is Health Financing?

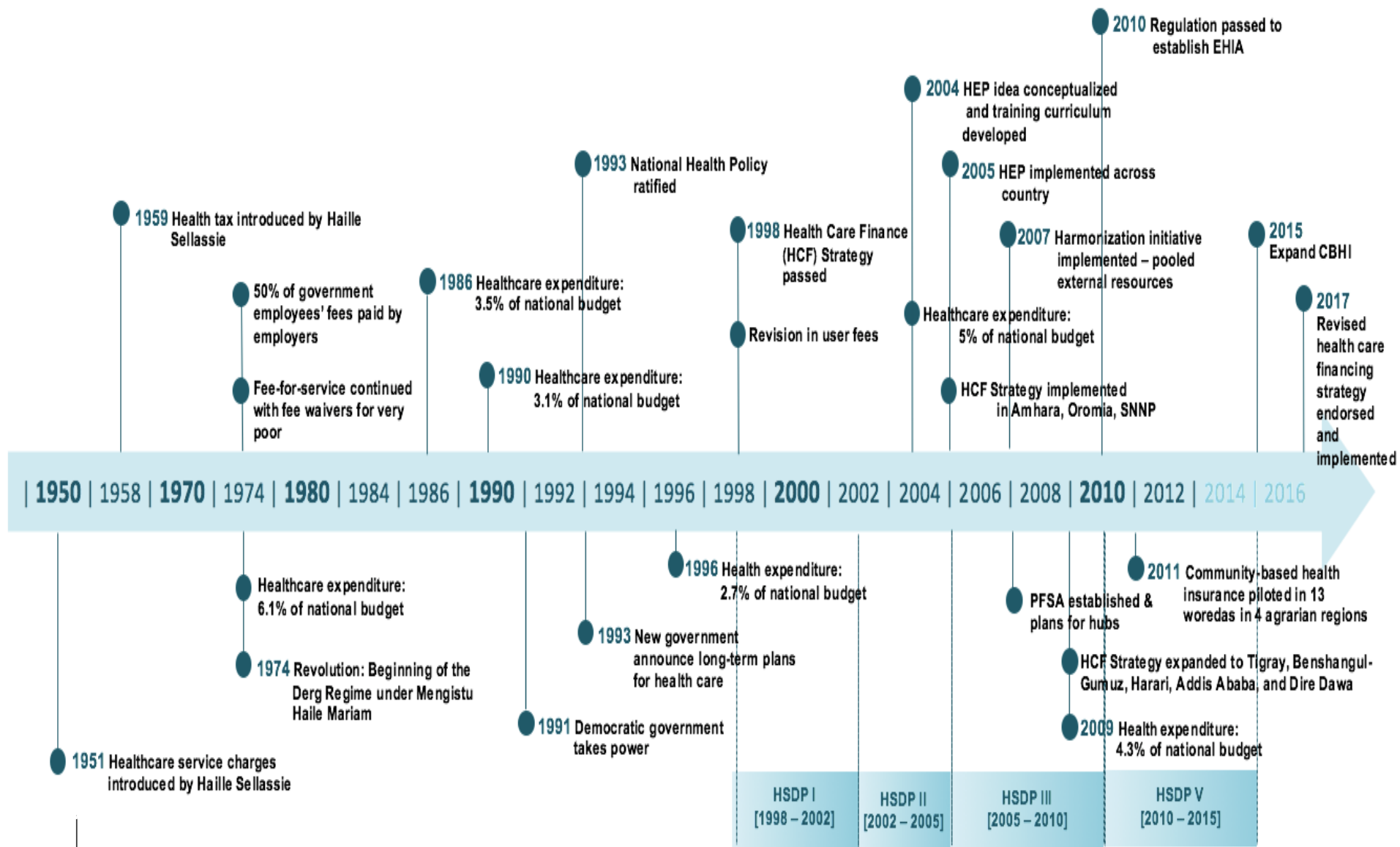
❑ **Narrow concept:** Financing
Defined as Mobilizing Financial
Resources



❑ **Broad Concept:** alternative
arrangements for paying, allocating,
organization and managing health
resources (as defined in HCF strategy)



The Evolution of Healthcare Financing in Ethiopia



Overview of the 1998 health care and financing strategy

- **The 1998 health care & financing goals were**
 - ❑ Identified and obtained resources from both **domestic and external resources**;
 - ❑ Increased **efficiency** in the use of available resources through allocating **more resources** to **primary care**;
 - ❑ Promoted **sustainability** of the health care financing;
 - ❑ Improved the **quality and coverage** of health services (council of minsters, 1998).

Strategy

- ☐ Improving Government Health Sector (allocative, therapeutic and operational) efficiency through improving allocation, organization and management of existing health resources.
- ☐ Generating Additional and New Sources of Revenue (government allocation, revision of user fees, revolving drug sales, various private, community, employer-based social financing and insurance plans)
- ☐ Encouraging Private / NGO participation
- ☐ Promotion of Community Participation
- ☐ Encouraging Bilateral and Multilateral Agencies Participation
- ☐ Developing Alternative Financing Options for urban areas like Addis Ababa to improve efficiency, quality and to release government resources

Principles

- ☐ Services will be offered on the basis of **cost-sharing between the receiver of services (patient) and provider of services (government)**; but patients are envisioned to pay according to their ability.
- ☐ **User fees** retained and used by the health facilities to improve the quality and quantity of services.
- ☐ **Fee waivers shall be granted** to reduce financial barriers for the poor, and an appropriate third party shall cover its cost.
- ☐ **Exemptions** given to encourage consumption of particular kinds of **preventive or public health services**.
- ☐ **Hospitals and health centers will have greater responsibility, authority, and accountability** in managing service delivery and the resources they retained through their own facility management boards.
- ☐ Opportunities **for innovative resource generating or cost saving schemes** (including establishment of a **private wing/wards** for those who can pay more and to outsource non-clinical services to the private sector) were created to hospitals and health centers.

Reforms

To operationalize the strategy, specific reform measures known as first and second generation reforms, were implemented

- **First generation reform**

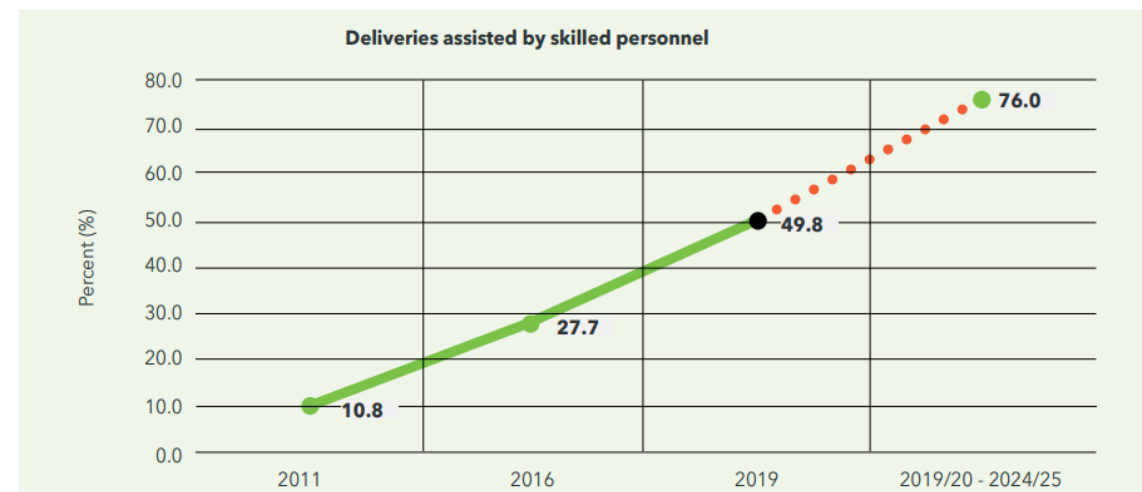
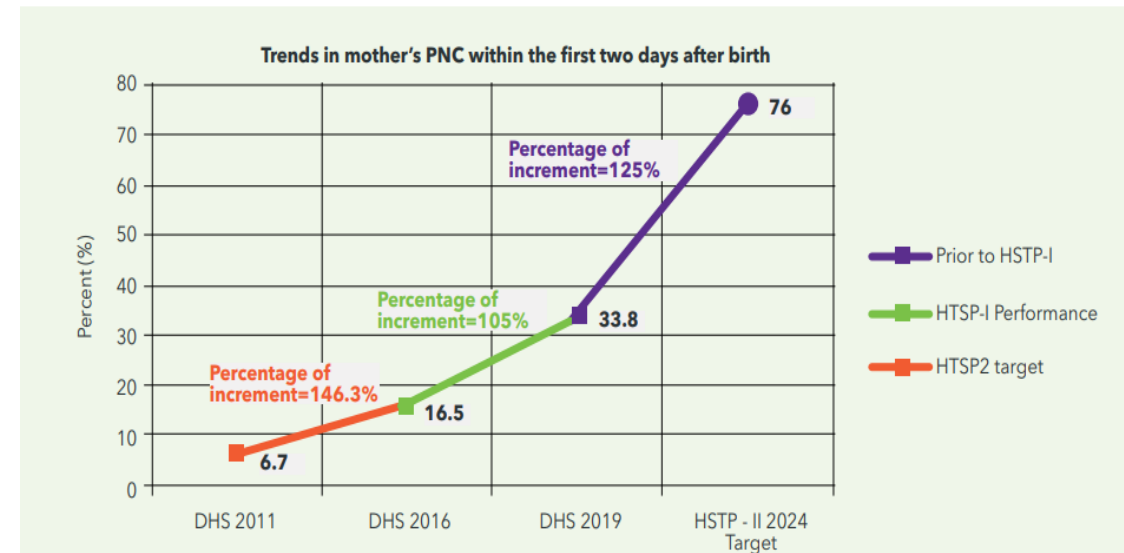
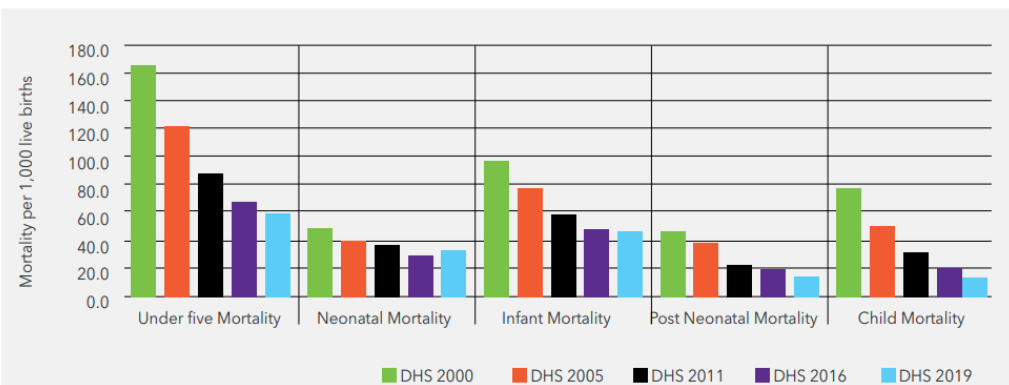
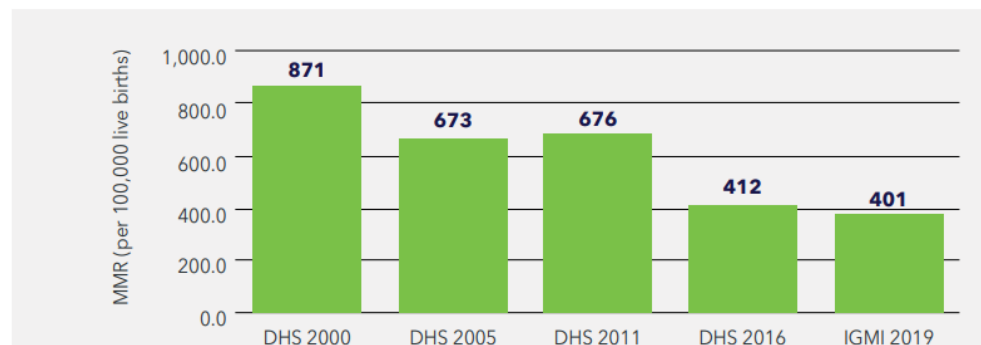
- ☐ Revenue retention and use at the health facility level;
- ☐ Systematizing the fee waiver system;
- ☐ Standardization of exempted services;
- ☐ Setting and revision of user fees;
- ☐ Allowing establishment of private wing in public hospitals;
- ☐ Promotion of health facility autonomy through establishment of a governance system.

- **Second generation reform**

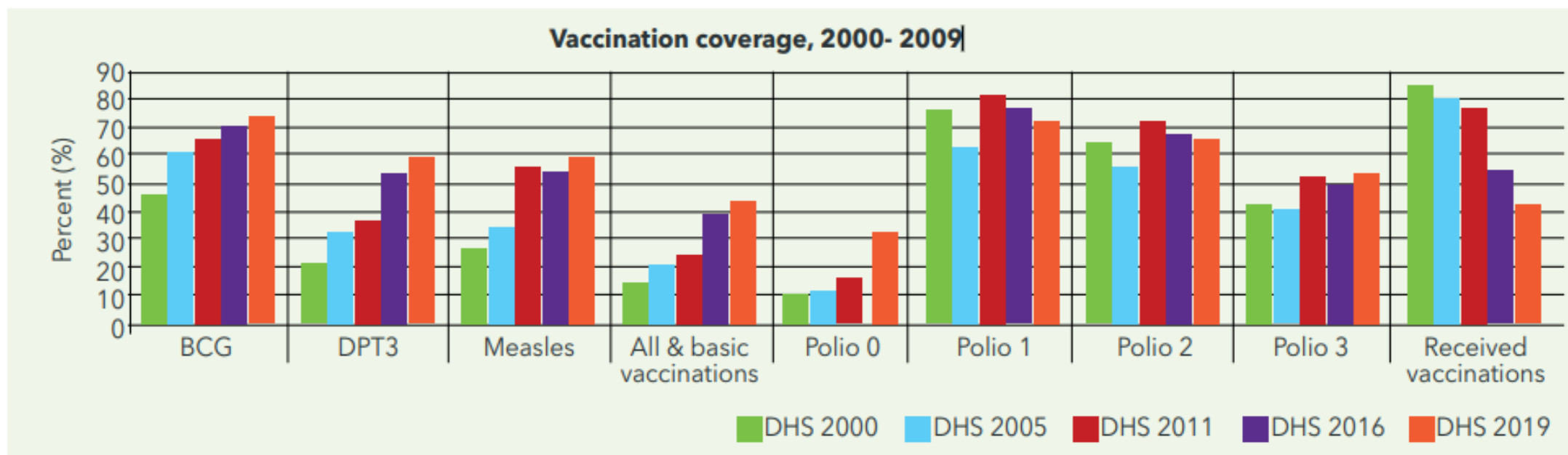
- ☐ The strategy has also laid foundation for the development of health insurance strategy that guided the design and implementation of health insurance activities such as CBHI and SHI.

Key Achievements and Challenges

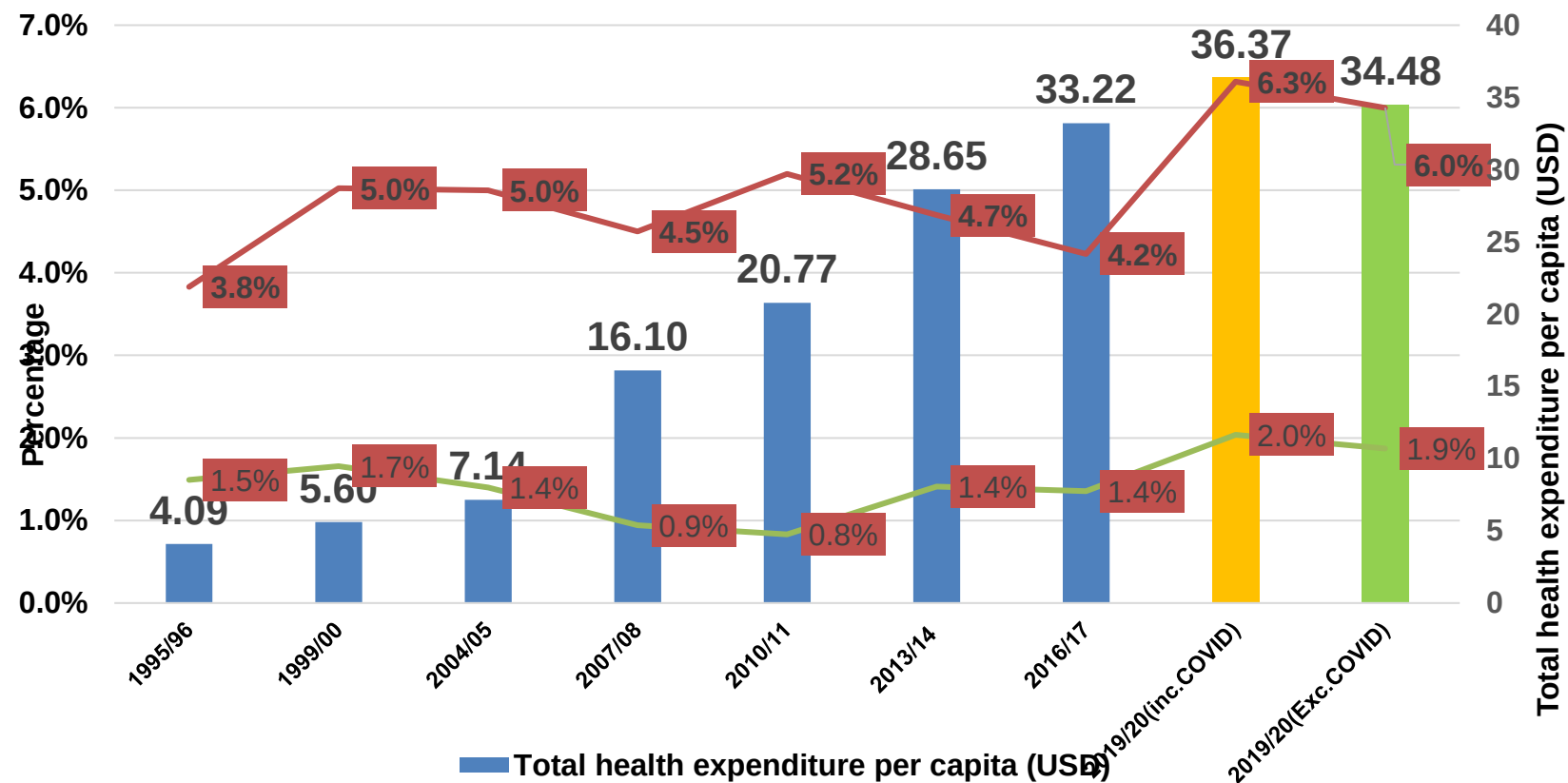
Achievement: Health Status Improved



Achievements

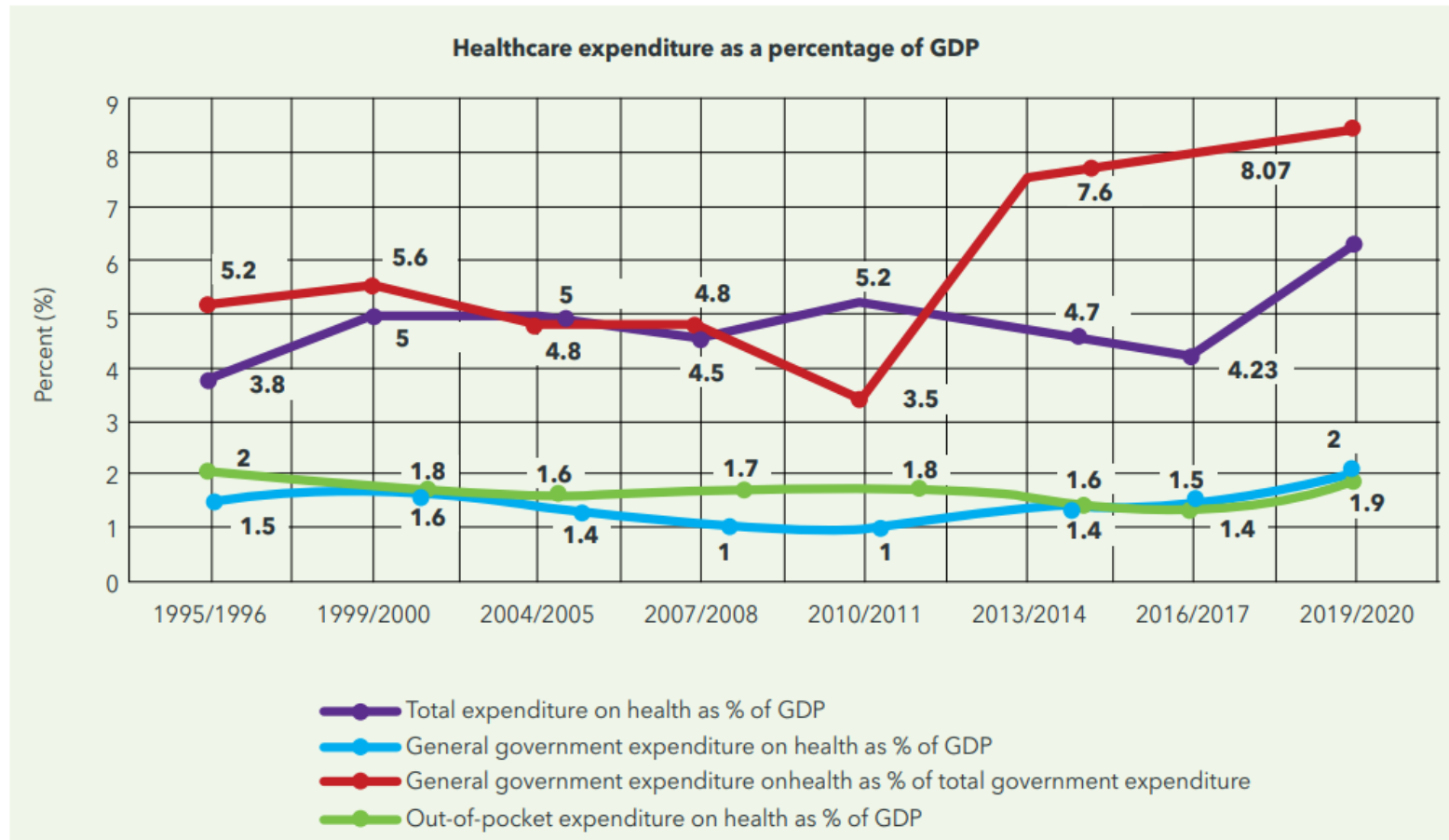


Key Achievements in Increasing Resource Mobilization



Total health expenditure per capita (USD), THE/GDP and THE/GGHE

Ethiopia's trends in per capita expenditure on health care per person, 2016-2018



Key Challenges

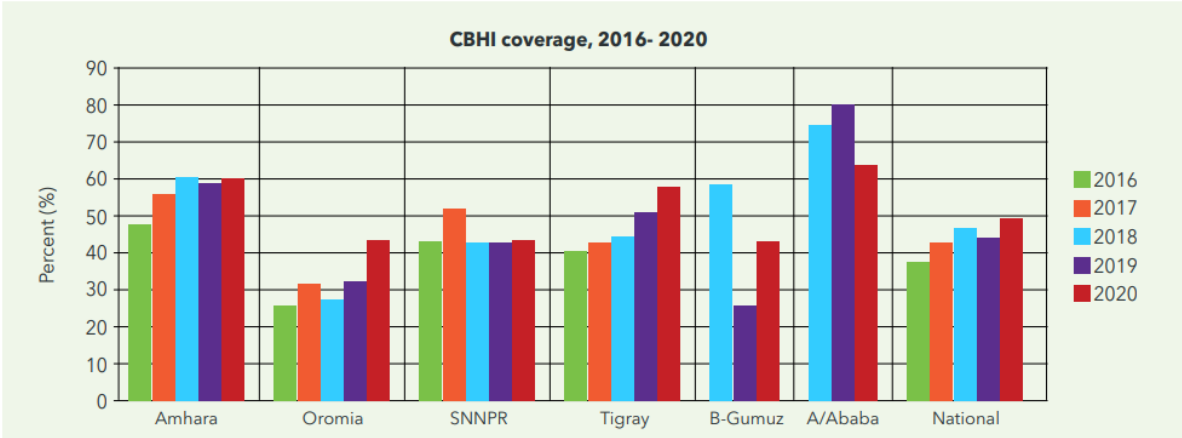
Increasing Resource Mobilization

Suggested by	Year	THE per capita	Objectives
CMH	2002	US\$38	To avail essential health services
	2012	US\$71	
HLTF for IF	2005	US\$54	To avail the more compressive health services.
	2012	US\$86	
WHO	2015	US\$86	
Government Financing	Health spending levels at least		
Abuja Target	2012	15% of govt spending	To be devoted in the health sector
WHO	2015	5% of GDP	To ensure basic PHC services

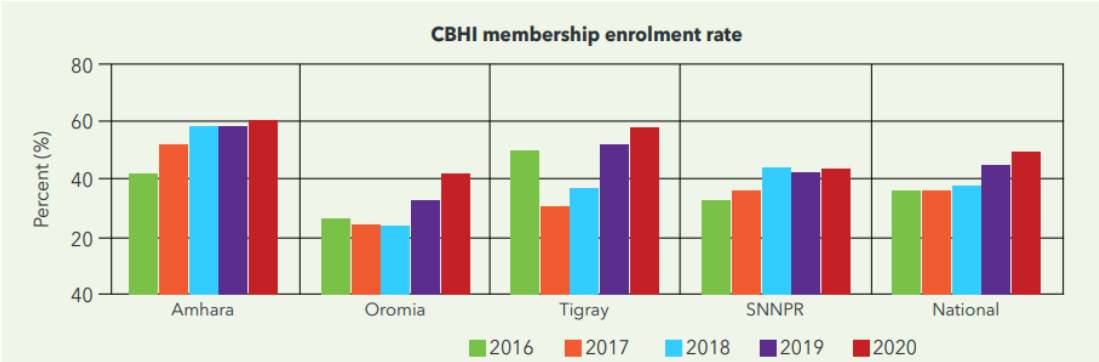
Key Achievements and Challenges

Achievement 1:Resource Mobilization Through CBHI Scheme (ETB)

CBHI Coverage by region (2016-2020)

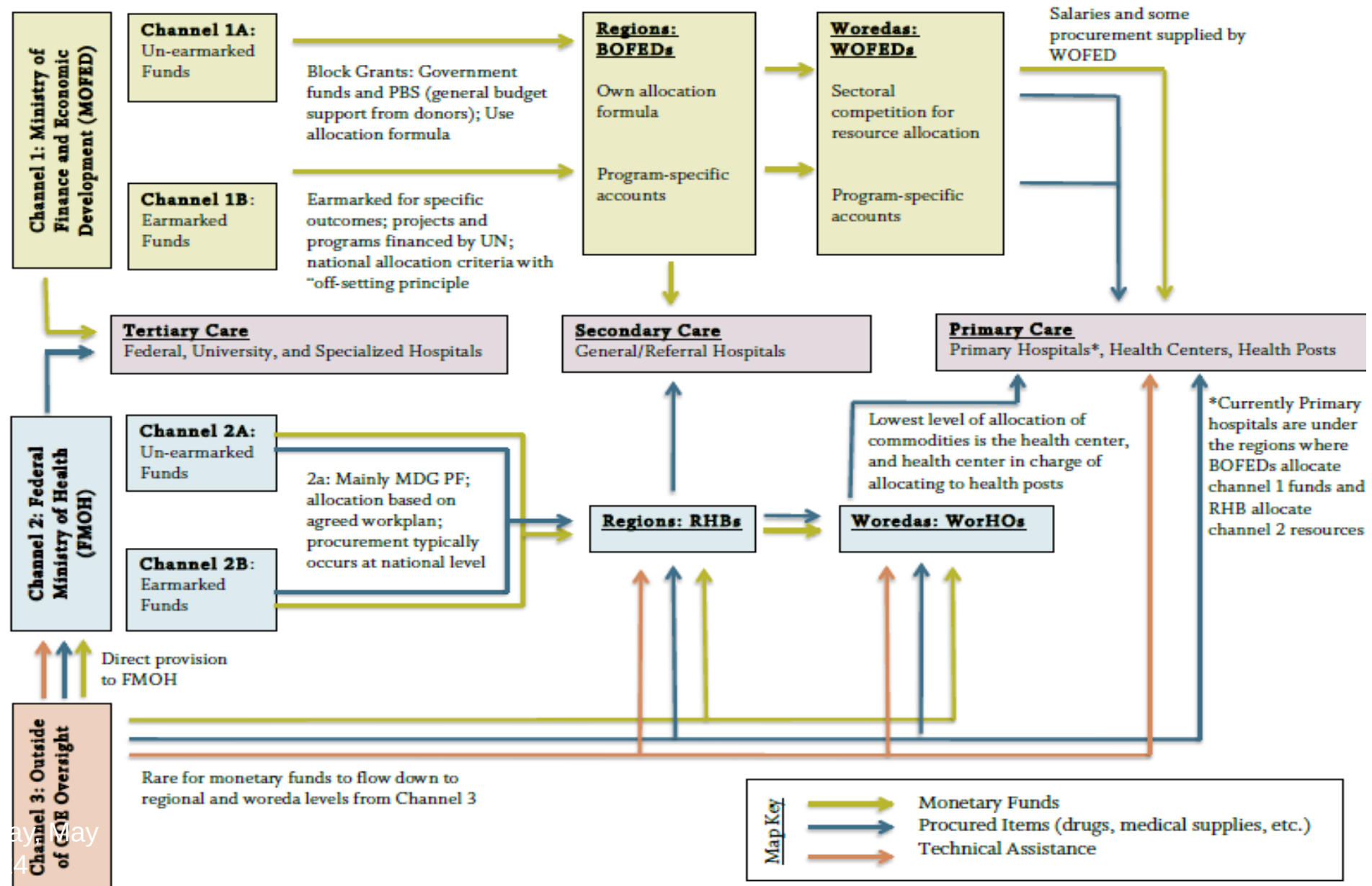


CBHI membership enrollment among regions that have started providing CBHI service by region (2016-2020)



Key Achievements and Challenges

Achievement 1 : Increasing External resource Mobilization, Alignment and Harmonization



Key Achievements and Challenges

Achievement 2 : Pooling

- ☐ Ethiopia has used four financing mechanisms to raise revenue, pool health risks, promote prepayment and purchase services:
 - ✓ **Government allocations** through ministries and RHBs;
 - ✓ **Development partners'** support;
 - ✓ **Community based health insurance** support;
 - ✓ Voluntary or private health insurance;
- ☐ MoH has developed and implemented a series of JFAs to insure harmonization and alignment of funding from external partners.
 - ✓ **International Health Partnership compact signed in 2008;**
 - ✓ **National Health Partnership compact signed in 2009;**
 - ✓ **The government and major development partners signed the first JFA in 2009, for 2009/10 to 2014/15, and the second JFA in 2015, valid until 2019/20;**
- ☐ JFAs facilitated a pooled funding mechanisms, MDG performance fund/SDG performance fund (in 2009 US\$900 million)
- ☐ **CBHI pools government contributions** with household contributions at Woreda level
- ☐ In 2008, MoH ratified the health insurance strategy and since 2010 **legal framework (proclamation & regulation) to implement SHI and establish EHIA**

Key Achievements and Challenges

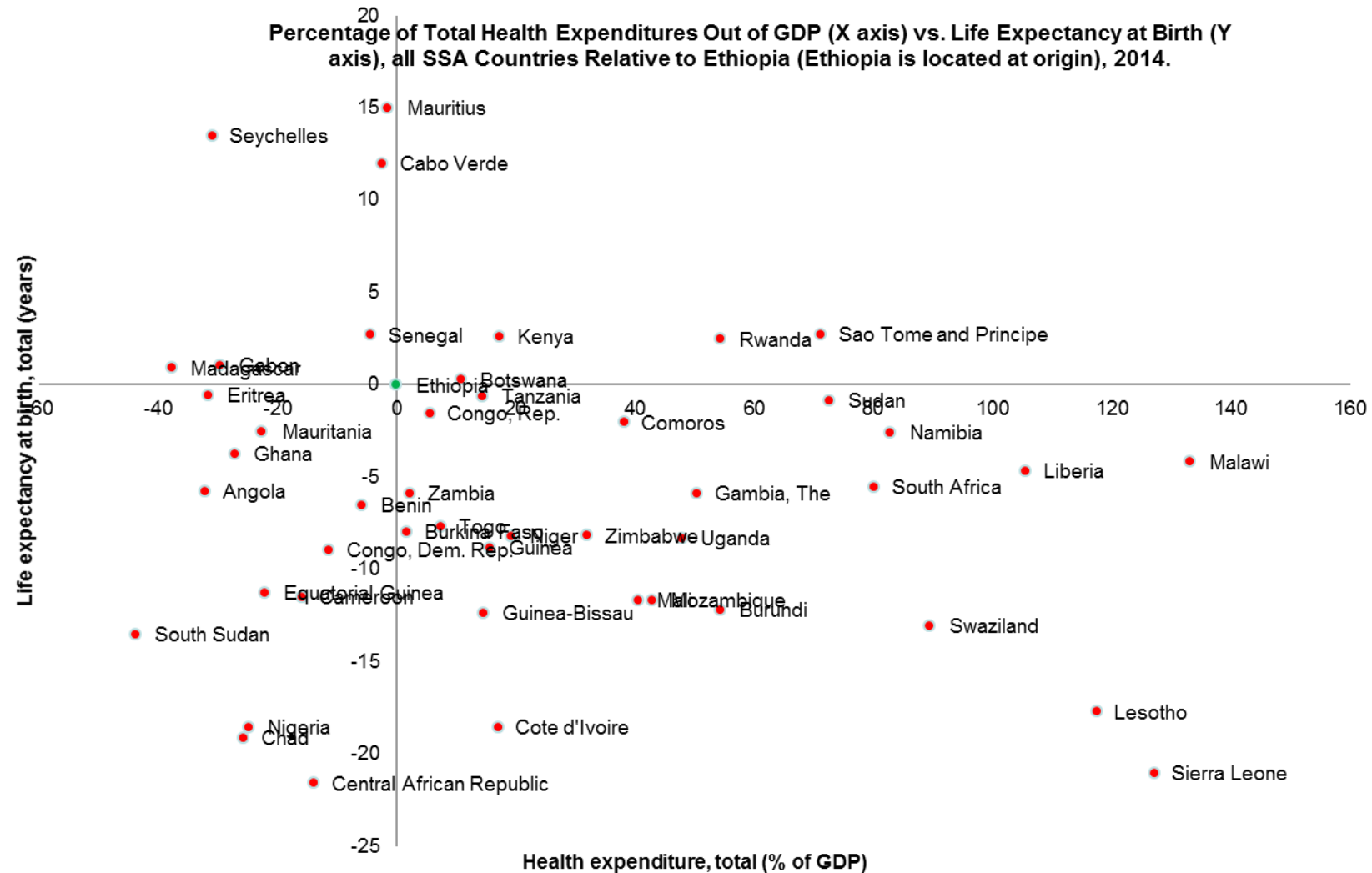
Achievement 3: Payment and purchasing mechanisms

☐ Main purchasers are

- ✓ MoH, RHBs, WorHOs, EHIA and other government entities that transfer funds to providers and health facilities to reimburse them for service delivery costs;
- ✓ Households are also main purchaser, especially via OOP spending on consultations, treatment, drugs, and tests;
- ✓ Public health facilities charge a considerable amount of in user fees, retain revenue and the fees waived for indigents;
- ✓ Exempted services: maternal and child health services (FP, ANC, delivery PNC and immunization) & Public health priorities (HIV, TB, and malaria);

Key Achievements and Challenges

Achievement 6: Enhancing Efficiency and Effectiveness of public expenditure on health



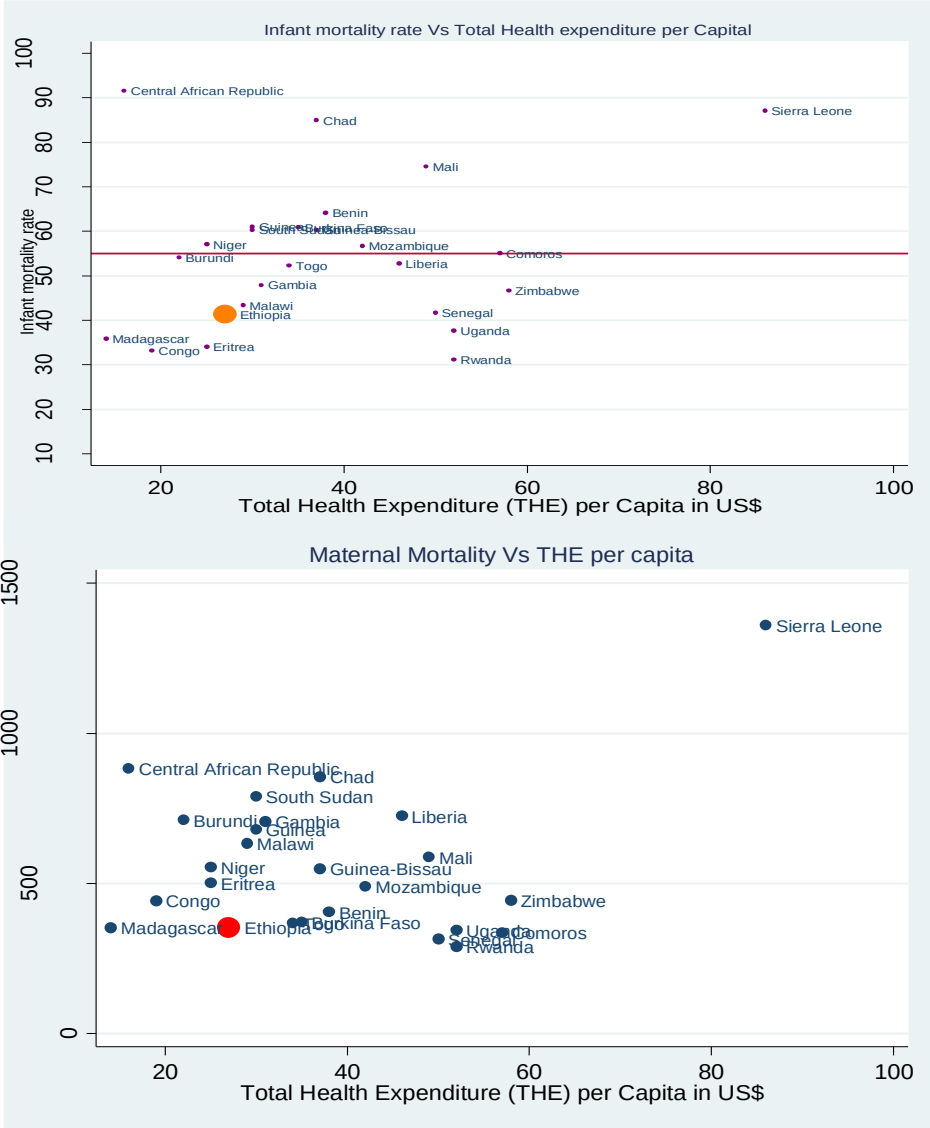
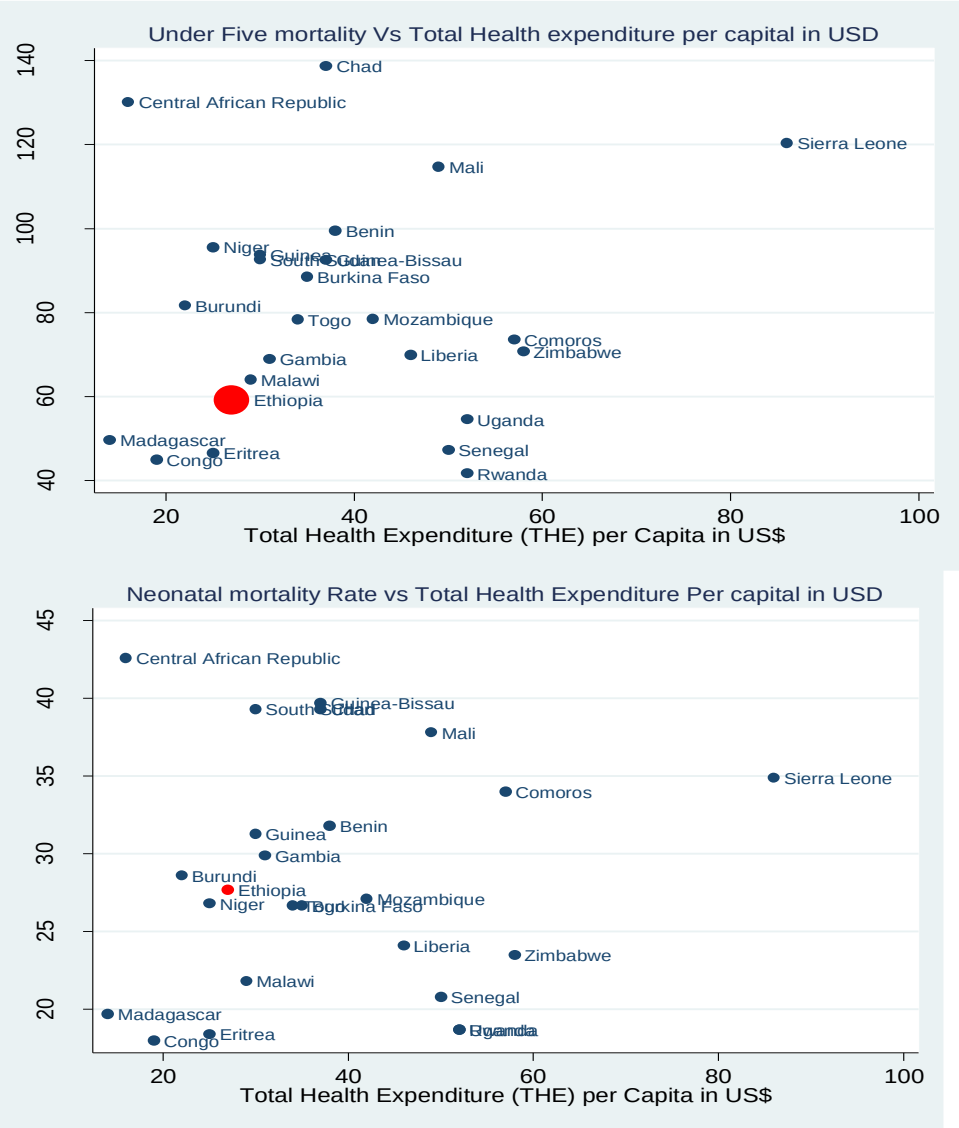
Key Achievements and Challenges

Achievement 6: Enhancing Equity in Resource Allocation and Utilization

- » There are inequalities in coverage of maternal health interventions in skilled birth attendance (SBA), ANC4+ and ANC1 visits between the richest and poorest sub-groups in 2019; interventions are concentrated among or are more utilized by the wealthy in the population.
- » Reducing child malnutrition is more pronounced on the richest and most privileged.
- » Vaccination is used more by the wealthiest segments of the population.
- » Reduction of Childhood malnutrition is more concentrated in the richest or more advantageous subpopulations. The reduction of stunting, wasting and underweight existed in 2000 and this inequalities more persisted in 2019.
- » Under-five mortality, infant mortality, stunting, wasting and underweight significantly increased as the wealth quintile decreased.

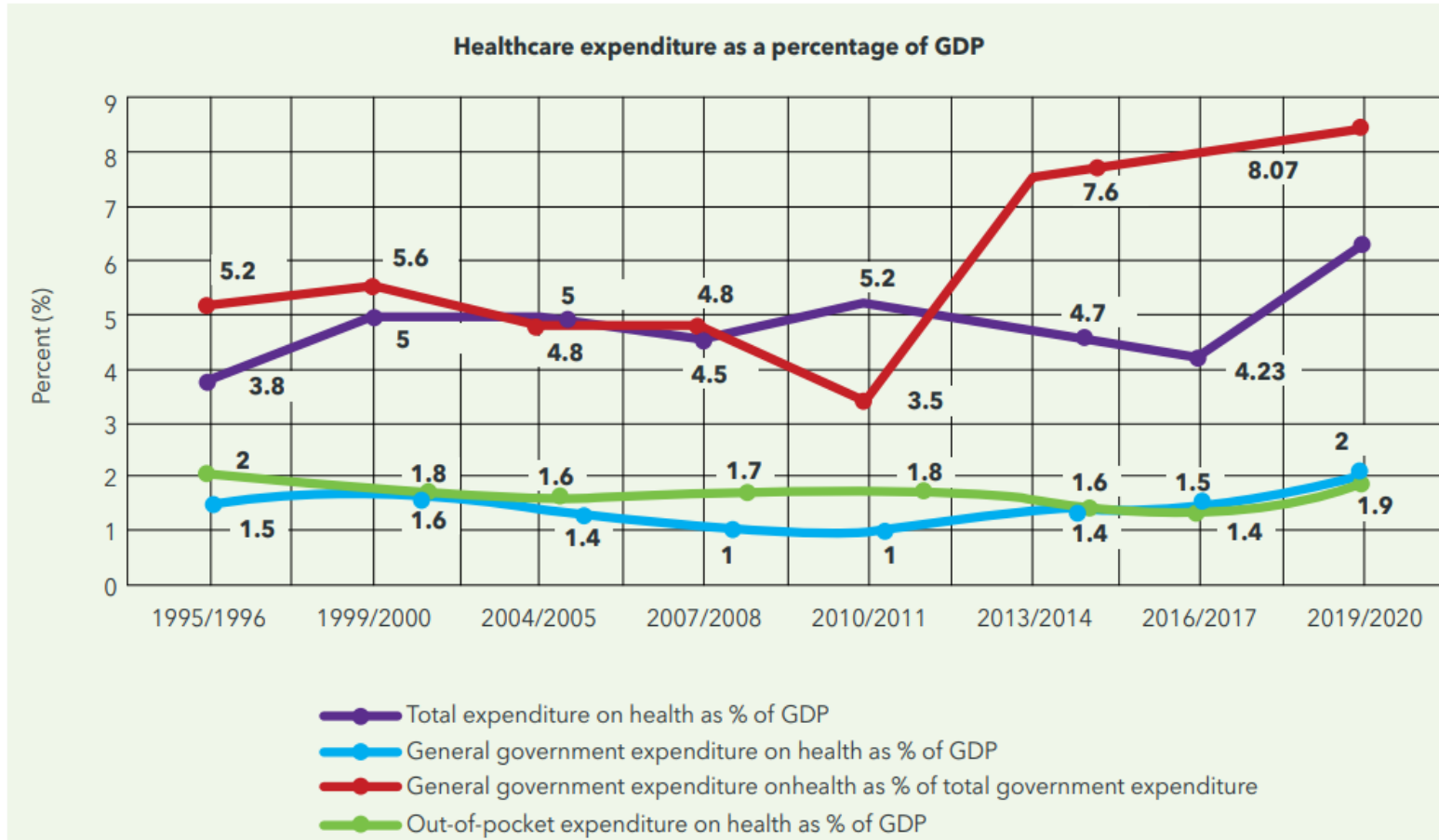
Key Achievements and Challenges

Achievement 6: Enhancing Efficiency and Effectiveness of public expenditure on health



Key Achievements and Challenges

Achievement 7: Fostering Sustainability



Key Achievements and Challenges.

Challenges

Challenge 1: Domestic Resource Mobilization

Challenge 2: Resource mobilization via fees and retention

- ❖ The implementation of reforms under the HCF strategy varies from region to region
- ❖ User fee revision has not occurred systematically across region
- ❖ Imbalance between federal function and expenditure assignment for health needs may lead to inadequate funding for exempted services
- ❖ There is divergence between HCF proclamation, regulations and guidelines and accrual implementation of the HCF strategy
- ❖ Federal level hospitals do not retain fees

Challenge 3: Progress in implementing and SHI

- ❖ Sustainability
- ❖ Narrow Fiscal Space and Resource scarcity
- ❖ Equity
- ❖ High OOP (Catastrophic effect of OOP and high likelihood to lead to impoverishment)

Challenge 4: External aid mobilization and utilization

Challenge 5: Efficiency and effectiveness

Challenge 6: Equity

Challenge 7: Capacity Development for HCF: Inadequate Institutional Capacity for implementing HCF

Lesson Learned

The 1998 Health care and Financing provides key lessons of the following

- ☐ Government ownership and institutional arrangement;
- ☐ Harmonized technical assistant;
- ☐ Timely generation and use of evidence for decision;
- ☐ Decentralized planning budget;
- ☐ Role of private sectors

Rational for Revising the Health care Financing Strategy.

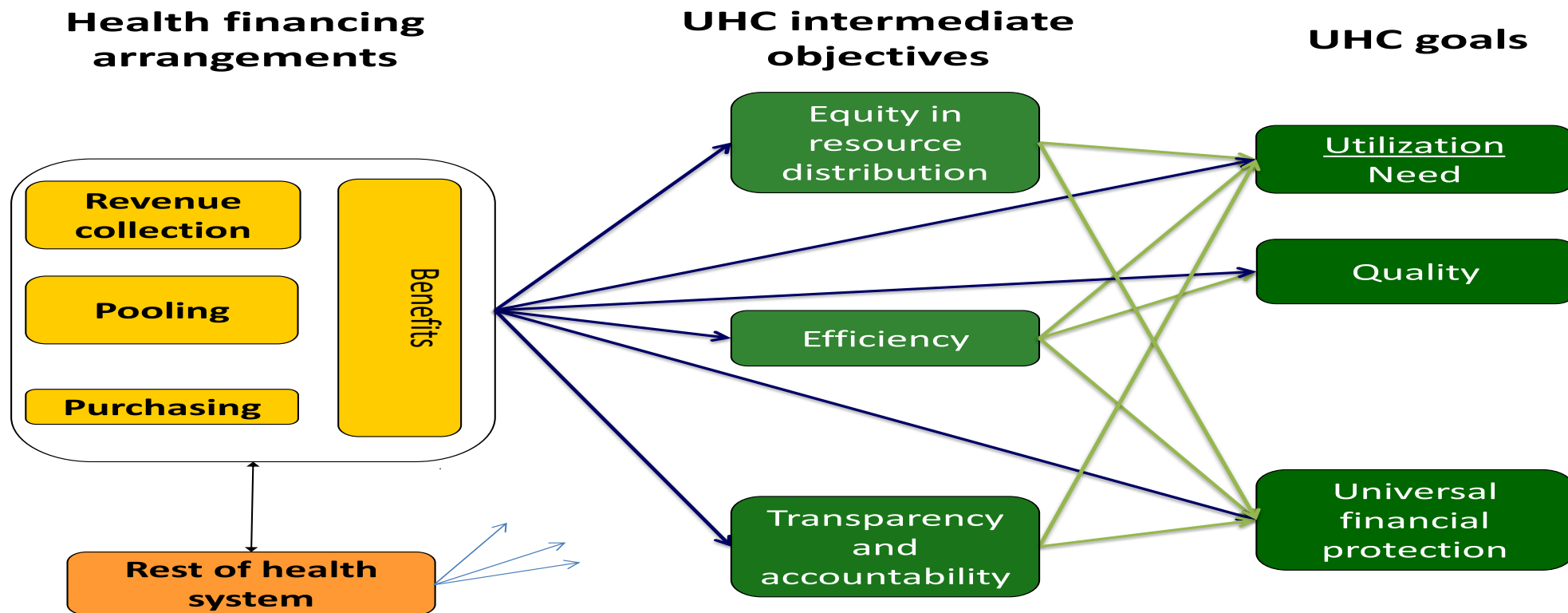
- ☐ Renewal focus on **UHC** and commitments of the **SDGs**;
- ☐ Anticipated effects of the expected economic growth on health and the health sector;
- ☐ Coping with the **local and international** aid dynamics e.g. **Trump Global Gag Rule**;
- ☐ Meeting the financing demand of epidemiological transition of disease;
- ☐ Technological advancement.

Introduction

- The government has also developed and implemented four successive five-year Health Sector Development Programs (HSDP) since 1997
- To attain the goals of the 1993 National Health Policy and the 1998 Health Care Financing strategy.
- The 1998 health care financing strategy emphasized on increasing the financial resources for health through a mixed approach with an objective of **improving service coverage and quality.**

Policy Orientation, Goal & Guiding Principles

UHC goals and intermediate objectives influenced by health financing



Policy Orientation, Goal & Guiding Principles (cont..)

Policy Orientation:

The 20-year plan, 'Envisioning Ethiopia's Path to Universal Health Coverage through strengthening of **Primary Health Care**' is a guiding document in developing the Health Care Financing Strategy.

Goal:

The health care financing strategy, in line with Ethiopia`s ambition to attain **universal health coverage** through primary health care such as **improving health status (utilizations), financial risk protection and public satisfaction (quality)**

Policy Orientation, Goal & Guiding Principles (cont..)

Guiding principles

I. Responsiveness:

- ❖ health financing interventions need to be responsive to changing epidemiological, demographic, economic, social, cultural, technological and environmental factors.

II. Sustainability:

- ❖ health financing interventions need to primarily rely on domestic means to ensure sustainability of financing and services
- ❖ while donors have an important role in complementing the country's effort to accelerate progress towards universal coverage.

Policy Orientation, Goal & Guiding Principles (cont..)

Guiding principles (cont..)

III. Financial risk protection:

- ❖ high reliance on out of pocket payments is associated with an increased risk of households being affected by financial catastrophe, being pushed in to poverty

IV. Equity:

- ❖ In designing alternative health financing options considerations should be made on how to raise funds equitably (**according to ability to pay**) and (**based on the needs of individuals**).

V. Efficiency:

- ❖ The health financing strategy should enable to provide high quality of care to the people among other measures by allocating resources efficiently.

Policy Orientation, Goal & Guiding Principles (cont..)

Guiding principles (cont..)

Vi. Public Satisfaction:

- ❖ Ethiopia has managed to establish a high degree of ‘trust’ between the public health service delivery system and its citizens, resulting in high usage of basic health services

Vii. Health in all policies:

- ❖ the healthcare financing strategy promotes working in collaboration and strengthen the systematic integration of health and its implications in other sector policies through multisector collaboration.

Strategic objectives and initiatives

The overall objective of this strategy

is to accelerate Ethiopia's progress towards Universal Health Coverage by defining a **'framework for strategic actions'** to enable Ethiopians to

- ☐ **To achieve the best health outcomes,**
- ☐ **Financial protection against catastrophic health illness, and**
- ☐ **Citizen satisfaction**

that would be comparable to lower middle income country by 2025, and an upper middle income country by 2035

Strategic objectives and initiatives (cont..)

Five strategic objectives and corresponding strategic initiatives are identified.

The five strategic objectives of the HF Strategy 2017 – 2025 are:

- ❖ **SO 1:** Mobilize adequate resources, through traditional and innovative approaches, from domestic and external sources
- ❖ **SO 2:** Reduce out-of-pocket spending at the point of use
- ❖ **SO 3:** Enhance efficiency and effectiveness
- ❖ **SO 4:** Strengthen public private partnership
- ❖ **SO 5:** Capacity development for improved health care financing

Strategic objectives and initiatives (cont..)

SO 1: Mobilize adequate resources, through traditional and innovative approaches, from domestic and external sources

Strategic initiatives

- I. Increase government budget allocation for health
- II. Generate additional finances from innovative financing mechanisms
- III. Mobilize donor support to ensure continued and aligned investment
- IV. Scale up the pre-payment mechanisms

Strategic objectives and initiatives (cont..)

SO 2: Reduce Out of Pocket (OOP) Spending at the point of use

Strategic initiatives

- I. Make health service fees affordable
- II. Strengthen the provision of exempted health services
- III. Strengthen Health Insurance System
- IV. Strengthen coverage for poor

Strategic objectives and initiatives (cont..)

SO 3: Enhancing efficiency and effectiveness

Strategic initiatives

- I. Continue to invest on high impact and cost-effective interventions
- II. Develop an effective harmonized financing and purchasing functions
- III. Improving allocative and operational efficiency
- IV. Enhance transparent and accountable resource utilization
- V. Assess, identify and implement different performance linked strategies to improve equity, efficiency and effectiveness

Strategic objectives and initiatives (cont..)

SO 4: Strengthening public-private partnership

Strategic initiatives

- I. Improving the enabling environment for Public Private Partnership for Health (PPPH) including establishment of simple and transparent partnership modalities; and,
- II. Accessing and scaling up of the on-going partnerships with private-for-profit and private-not-for-profit actors.

Strategic objectives and initiatives (cont..)

SO 5: Capacity development for improved health care financing

Strategic initiatives

- I. Improve the health sector's / organizational capacity for Health Care Financing at all administrative, management and delivery levels by improving coordination between all stakeholders;
- II. Build capacity to improve transparency and accountability of the sector;
- III. Build capacity to identify and apply lessons, local and international evidence.

Governance Arrangement

The major actors of the HCF strategy are policy makers and administrators at

- ✓ Federal Ministry of Health,
- ✓ Regional Health Bureaus,
- ✓ Woreda Health Offices,
- ✓ Public and Private Health Facilities,
- ✓ Health Insurance Agency and its branches,
- ✓ Ministry of Social and Labor Affairs,
- ✓ Federal Customs and Revenue Collection Authorities,
- ✓ Ministry of Finance and Economic Cooperation and their counter parts at regional and woreda levels,
- ✓ Civil Society organizations and International and National Development Agencies/Development Partners.

Governance Arrangement (cont...)

Strategic approaches

- I. Improve stakeholders coordination and oversight in health financing at different levels
- II. Strengthen the National and Sub national level structures for improved health care financing implementation

Monitoring and Evaluation

S.N	Health financing performance indicator	Guidance (indicative targets)	Baseline (Year)	Target (2020)	Target (2025)	Source of information
Level of funding						
1	Total Health Expenditure (THE) per Capita in USD	Estimates	28.65 (2013/14)	42	60	NHA report
2	General Government Health Expenditure (GGHE) per Capita in USD	Estimates	8.60 (2013/14)	13.44	20.4	NHA report
3	General Government Health Expenditure (GGHE) % General Government Expenditure (GGE) (excluding external resources)	Government Expenditure on health ÷ total government Expenditure	6.65% (2013/14)	7.00%	8.50%	NHA report
Level of population coverage						
4	Proportion of population covered in prepayment schemes	Membership of SHI, CBHI and private health insurance/total population	1.2% (2014)	20%	40%	Admin Report
5	Access to and utilization of health care	Utilization of overall health services per person per year	0.56 (2016)	0.8	1.5	NHA/Admin report
Degree of financial risk protection						
6	Out of Pocket Expenditure (OOPS) as % of Total Health Expenditure (THE)*	OOP ÷ THE	34% (2013/14)	27%	20%	NHA report
7	Incidence of catastrophic health expenditures (% of households which have experienced catastrophic health expenditures)	Estimates	3% (2015/16)	2.50%	2%	NHA/WMS report
Level of pooling across the health financing system						
8	% of donor funds pooled (in SDG PF)	Donor harmonization	38% (2015/16)	50%	60%	Admin Report

Thank you !!